

MassEnviroScreen Documentation

UPDATED: July 30, 2025

Introduction

This document presents a tool for identifying or prioritizing the most environmentally vulnerable or burdened communities in Massachusetts based on a cumulative impact score that incorporates exposure to pollution and climate risks and the presence of sensitive or vulnerable populations, as well as whether the community meets criteria related to limited English proficiency, household income, or presence of tribal land. It is intended to act as a supplement to the Massachusetts Environmental Justice Population definition to identify areas that experience the highest burden and vulnerability.

This tool - MassEnviroScreen - is a hybrid approach that utilizes elements of the current [Massachusetts Environmental Justice Population definition](#), the presence of [federally recognized tribal lands](#), health thresholds, and a cumulative impact mapping approach. The cumulative impact mapping approach is modeled on those used by similar tools across the country, especially [California EPA's CalEnviroScreen tool](#) and the [Colorado EnviroScreen tool](#). These tools utilize a 'cumulative impact score' to describe the relative environmental burden of communities across the state and to prioritize those that are most burdened. California defines cumulative impacts as “the exposures, public health or environmental effects from the combined emissions and discharges, in a geographic area, including environmental pollution from all sources, whether single or multi-media, routinely, accidentally, or otherwise released.” Impacts consider “sensitive populations and socio-economic factors, where applicable and to the extent data are available.” The Colorado tool augments the CalEnviroScreen approach by adding climate risks, which is the approach followed here.

A 'cumulative impact score' is a numerical value that ranks every community (i.e., census block group) on a scale from 0 to 100. Higher values indicate greater cumulative burden. These values also represent percentile ranks, which means that a community's score also indicates the percentage of scores in a group that are equal to or lower than a given score. For example, a census block group with a score of 50 (50th percentile) means that its cumulative impact score is equal to or higher than 50% of census block groups in the state. In this model, we follow California's example of using a score of 75 (the 75th percentile) as one of the thresholds for identifying the most impacted or 'Unfairly Burdened Areas.' Census block groups with a minimum score of 75 experience cumulative impacts that are

equal to or higher than 75% of block groups in the state. In other words, these block groups represent the top 25% of cumulative impact scores in Massachusetts.

To define “Unfairly Burdened Areas” in this tool, cumulative impact scores of 75 or higher are combined with several other criteria: two of the [existing Massachusetts EJ criteria](#) - limited English speaking households and low income, the presence of federally-recognized tribal territory, and communities where health or air quality indicators exceed 200% of the statewide average.

Unfairly Burdened Areas are communities (i.e., census block groups) that meet one or more of the following criteria:

- cumulative impact score (i.e., MassEnviroScore) of 75 or greater, OR
- annual median household income is 65 percent or less of the statewide annual median household income, OR
- 25 percent or more of households identify as speaking English less than “very well”, OR
- occupy or intersect land within the boundaries of Federally Recognized Tribes OR
- health or air quality indicators exceed 200% of the statewide average

MassEnviroScreen scoring methodology

The MassEnviroScreen cumulative impact score model is based on three components representing Pollution and Climate Burden – Exposures and Environmental Effects and Climate Risks – and two components representing Population Characteristics – Sensitive Populations (e.g., in terms of health status and age) and Socioeconomic Factors.

Model characteristics

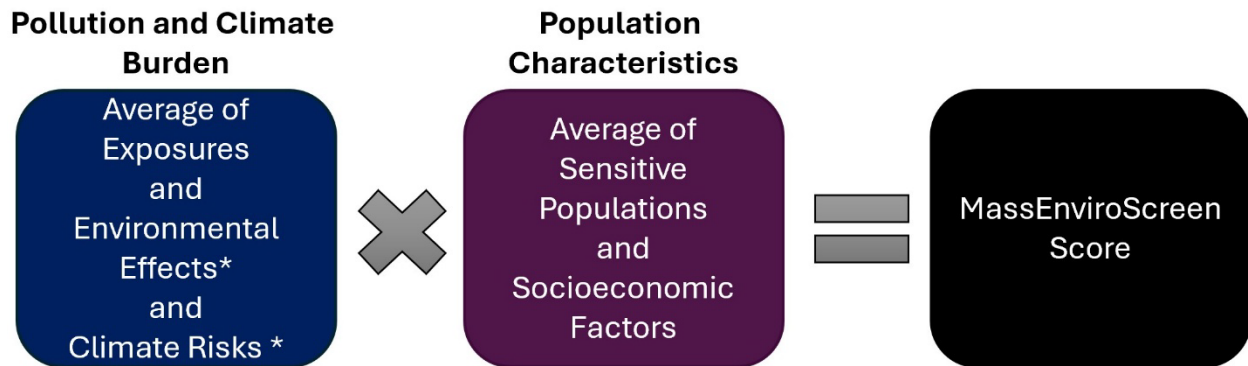
The model:

- Uses 30 statewide indicators to characterize both Pollution and Climate Burden and Population Characteristics
- Uses percentiles to assign scores for each of the indicators in a given geographic area. The percentile represents a relative score for the indicators.
- Uses a scoring system in which the percentiles are averaged for the set of indicators in each of the four components (Exposures, Environmental Effects, Climate Risks, Sensitive Populations, and Socioeconomic Factors).

- Combines the component scores to produce a MassEnviroScreen score for a given place relative to other places in the state, using the formula below.

Formula for calculating MassEnviroScreen Score

After the components are scored within Pollution Burden and Climate Risk or Population Characteristics, the scores are combined as follows to calculate the overall MassEnviroScreen Score:



* The Environmental Effects and Climate Risks scores were weighted half as much as the Exposures score.

Scores for the pollution burden and climate risk and population characteristics categories are multiplied (rather than added, for example). This multiplication approach is based on research which shows that socioeconomic and health/sensitivity factors are “effect modifiers” that multiply the risks posed by pollutants and climate risks. Various environmental health and emergency response organizations use a similar scoring system: *Risk = Threat x Vulnerability*.

Within the MassEnviroScreen score map, clicking on individual census block groups shows both the final MassEnviroScreen score as well as the Pollution and Climate Burden and the Sensitive Pop SubScores. The Pollution and Climate Burden Subscore represents the percentile rank (0 - 100) of the block group for averaged pollution burden and climate risk component indicators. Similarly, the Sensitive Pop SubScore represents the percentile rank (0 - 100) of the block group for averaged population characteristics component indicators.

Model indicators

The MassEnviroScreen score model is computed from 30 statewide environmental, socioeconomic, and health indicators. These indicators are listed in Table 1 below. Details and data sources for model indicators are described later in this document.

Table 1: MassEnviroScreen Score Indicators

Pollution and Climate Burden

Population Characteristics

Environmental Exposures	Environmental Effects	Climate Risks	Sensitive Populations	Socioeconomic Factors
PM2.5	Pollution Cleanup Sites	Drought	Pediatric Asthma	Adults without a High School degree
Ozone	Groundwater Threats	Wildfire Risk	Low Birth Weight Infants	Housing Burdened Low Income Households
Nitrogen dioxide (NO2)	Hazardous Waste Generators and Facilities	Flood Risk	Elevated Blood Lead Levels in Children	Linguistically Isolated Households
Diesel Particulate Matter	Solid Waste Sites and Facilities	Extreme Heat Days	Premature Mortality	Poverty
Drinking Water Non-compliance	Impaired Water Bodies		Adult High Blood Pressure	Unemployment
Air Toxics Cancer Risk			Coronary Heart Disease	
Respiratory Hazard Index			Chronic Obstructive Pulmonary Disease	
Proximity to Heavy Traffic			Adult Cancer	

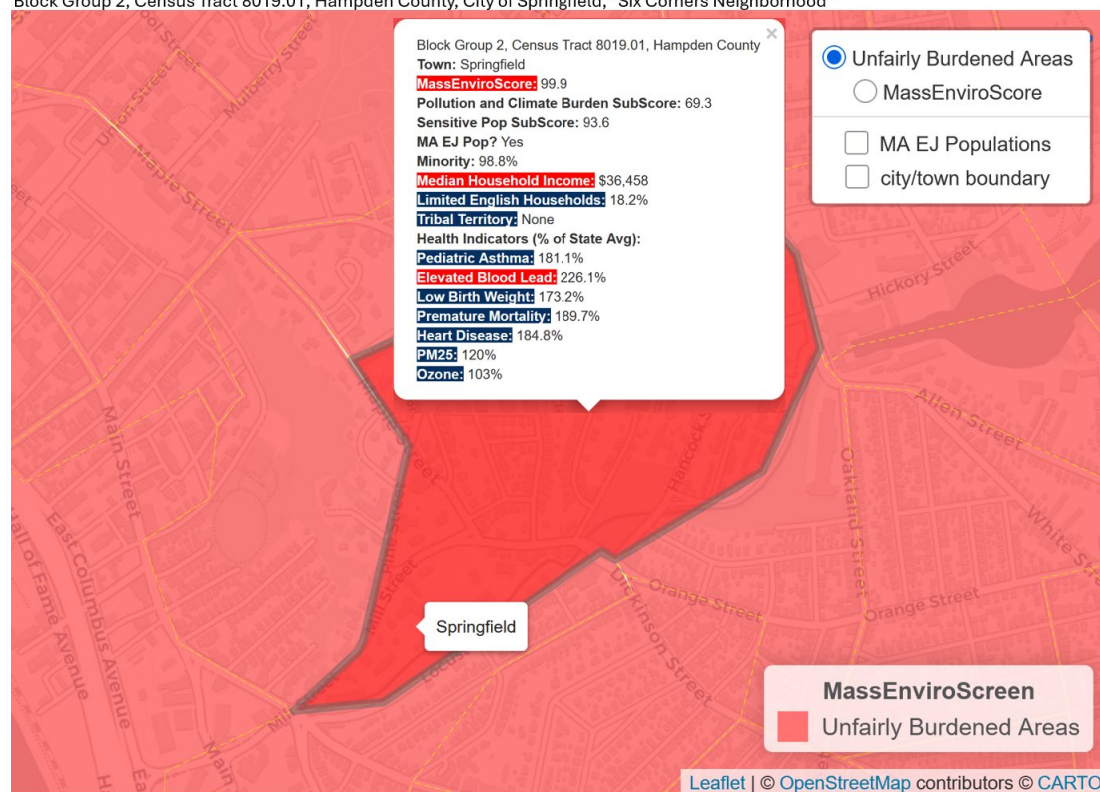
Example Census Block Group: Indicator Results, MassEnviroScreen Score, and designation as Unfairly Burdened Area

One example census block group in Springfield was selected to illustrate how an overall MassEnviroScreen score is calculated and how a community is designated as an Unfairly Burdened Area (UBA).

Shown below are:

- A map of a census block group in the Six Corners neighborhood of Springfield, Massachusetts (highlighted in dark red). The block group GEOID is 250138019012.
- Tables for the indicators of Pollution and Climate Burden and Population Characteristics with raw and percentile scores for each of the indicators.
- A table showing how a MassEnviroScreen score was calculated for this block group.

Block Group 2, Census Tract 8019.01, Hampden County, City of Springfield, "Six Corners Neighborhood"



Block Group 2, Census Tract 8019.01, Hampden County, City of Springfield, "Six Corners Neighborhood"

Exposure Indicators	Raw Value	Percentile
PM2.5	7.83 µg/m³	99
Ozone (O ₃)	58.39 ppb	80
Nitrogen Dioxide (NO ₂)	13.06 ppb	84
Diesel Particulate Matter	0.27 µg/m³	84
Drinking Water Non-Compliance (index)	6	77
Air Toxics Cancer Risk	30	54
Respiratory Hazard Index	0.3	59
Proximity to Heavy Traffic (impact index)	4,650,593	53
AVERAGE COMPONENT SCORE		73.8

Effects Indicators	Raw Value	Percentile
Pollution Cleanup Sites (weighted count)	66	98
Groundwater Threats (weighted count)	0	0
Hazardous Waste Generators and Facilities (weighted count)	0	0
Solid Waste Sites and Facilities (weighted count)	0	0
Impaired Water Bodies (count of pollutants)	2	65
AVERAGE COMPONENT SCORE		32.5

Climate Risks	Raw Value	Percentile
Drought (index)	1,988.7	23
Wildfire Risk (index)	6	60
Flood Risk	0%	0
Extreme Heat Days (average number per summer)	21.8	89
AVERAGE COMPONENT SCORE		43

Block Group 2, Census Tract 8019.01, Hampden County, City of Springfield, "Six Corners Neighborhood"

Sensitive Populations	Raw Value	Percentile
Pediatric Asthma (prevalence per 100 students K-8)	22.1%	96
Adult High Blood Pressure (prevalence)	38.8	97
Adult Cancer (prevalence)	3.9	4
Coronary Heart Disease (prevalence)	8.5	88
Low Birth Weight Infants (percent of live births)	3.8%	88
Chronic Obstructive Pulmonary Disease (prevalence)	10.4	97
Elevated Blood Lead Levels in Children (prevalence per 1,000 children 9 – 48 mos)	41.6	94
Premature Mortality (per 100,000 residents)	554.7	95
AVERAGE COMPONENT SCORE		82.4

Socioeconomic Factors	Raw Value	Percentile
Adults without a High School Degree	40.7%	98
Housing Burdened Low Income Households	35.7%	95
Linguistic Isolation	18.2%	89
Poverty	42%	97
Unemployment	15.4%	94
AVERAGE COMPONENT SCORE		94.7

	Pollution& Climate Burden			Population Characteristics	
	Exposure	Environmental Effects	Climate Risks	Sensitive Populations	Socioeconomic Factors
Component Score (average of scaled indicators)	73.8	(0.5 × 32.5) = 16.3	(0.5 × 43) = 21.5	82.4	94.7
Average of Component Scores	111.6 ÷ 2 = 55.8 Pollution & Climate Burden is calculated as the average of its three component scores, with the Environmental Effects and Climate Risks components half-weighted			177.1 ÷ 2 = 88.6 Population Characteristics is calculated as the average of its two component scores	
Scaled Component Scores	(55.8 ÷ 80.5) × 10 = 6.9 The Pollution & Climate Burden percentile is rescaled by the statewide maximum Pollution Burden scores			(88.6 ÷ 94.6) × 10 = 9.4 The Population Characteristics percentile is scaled by the statewide maximum Population Characteristics scores	
MassEnviroScreen Score	6.9 × 9.4 = 64.9 A score of 64.9 puts this census block group in the 99th percentile or top 1% of all <u>MassEnviroScreen</u> scores statewide				

One of the thresholds for designation as a UBA is a MassEnviroScreen cumulative impact score equal to or greater than the 75th percentile for the state. At the 99th percentile, this census block group is well above that threshold and therefore qualifies as a UBA.

A census block group may also qualify as a UBA if it meets any of the following criteria:

- ☒ MassEnviroScreen cumulative impact score of 75 or greater, OR
- ☒ annual median household income is 65 percent or less of the statewide annual median household income, OR
- ☐ 25 percent or more of households identify as speaking English less than “very well,” OR
- ☐ the census block group occupies or intersects land within the boundaries of Federally Recognized Tribes, OR
- ☒ health or air quality indicators exceed 200% of the statewide average

In addition to the MassEnviroScreen cumulative impact score, this census block group meets the criteria for household income, and it also meets one of the health criteria because elevated blood lead levels are more than twice (226%) the state average. Although a census block group need only exceed one of these thresholds to qualify as a UBA, this example block group in Springfield exceeds three of those thresholds – 75th percentile MassEnviroScreen cumulative impact score, low median household income, and high percentage of children with elevated blood lead levels.

MassEnviroScreen Indicator Descriptions

For each indicator, this section provides the following details: Indicator name, definition, short description, which MassEnviroScreen component group it contributes to, the rationale (why it was selected as an indicator), the method (outlining which assumptions have been made within data processing), years or time period of data source, the original geographic unit or spatial scale of the data, and the data source. For more information about the original datasets, refer to the data source website/s included in each description.

Pollution and Climate Burden - Environmental Exposures

PM2.5

Definition: Average annual 24-hour average concentration of particulate matter that is less than or equal to 2.5 micrometers in diameter (PM_{2.5}) measured in micrograms per cubic meter (µg/m³).

Description: PM2.5 is a name for tiny particles (less than 2.5 micrometers in diameter) that can harm the heart and lungs. These particles come from many different sources. Sources include vehicle tailpipes, smokestacks, dust from construction sites, fires, and chemical reactions in the atmosphere.

Indicator type: *Environmental Exposures*

Rationale: PM2.5 is an air pollutant that has been associated with the largest health burden worldwide. Of the six criteria air pollutants regulated under the Clean Air Act, particulate matter and ozone pose the most widespread and significant health threats. PM2.5 is a group of particles from different sources and chemical components. Due to their small size, they have the capacity to be inhaled, travel to the lower respiratory tract, cross the lung-blood barrier, and be transported through the blood to any organ and system in the human body.ⁱ PM2.5 has been associated with multiple diseases and causes of death in adults and children. PM2.5 has the capacity to produce short and long-term health outcomes. In the short term, PM2.5 has been linked to pneumonia, cardiac arrhythmias, cardiac arrest, and mortality. In the long-term, PM2.5 exposure in children has been associated with asthma and autism spectrum disorder; PM2.5 exposure in pregnant women has been associated with adverse pregnancy outcomes; and PM2.5 exposure in adults has been associated with Alzheimer's disease, chronic kidney disease, chronic obstructive pulmonary disease, dementia, depression, ischemic heart disease, lung cancer, liver cancer, colorectal cancer, respiratory disease, stroke, type 2 diabetes, Parkinson's disease, and mortality, among others.ⁱⁱ

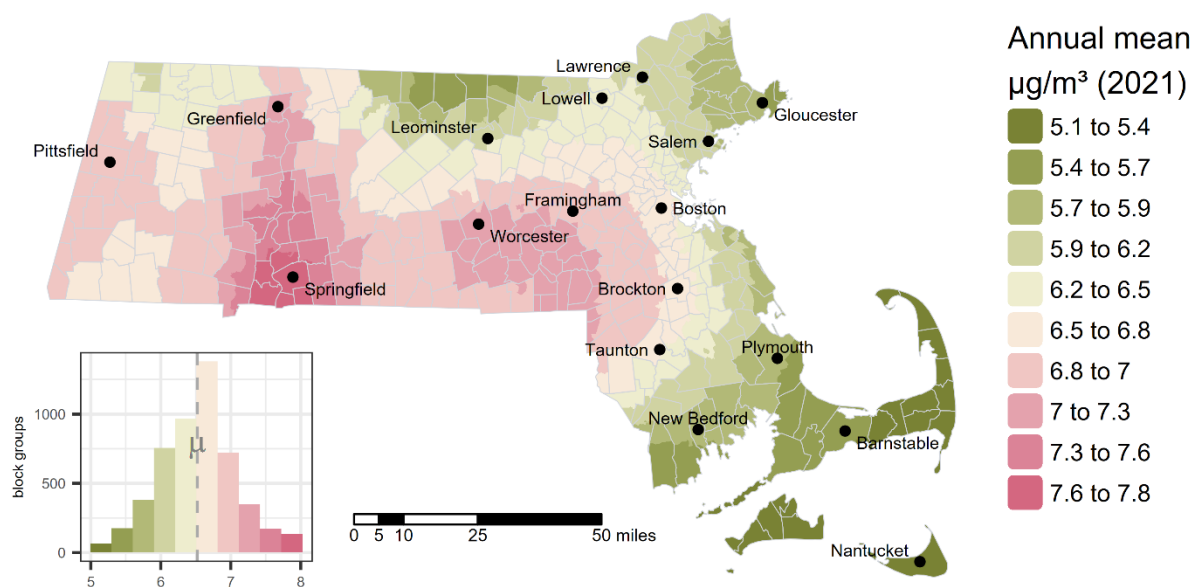
Method: PM_{2.5} air quality monitoring data from the State and Local Air Monitoring Stations (SLAMS) and numerical output from the Community Multiscale Air Quality (CMAQ) model were both input into a Bayesian Space-time Downscaling Fusion Model (Downscaler) to predict 2021 concentrations at the 2020 US census tract centroids encompassed by the CMAQ modeling domain. The mean values of all census tracts within a county were used to define the county PM_{2.5} measure. All census block groups received the same PM_{2.5} measure as the census tract in which they are contained.

Data years: 2021

Originating geographic scale of data: census tracts

Source: U.S. Environmental Protection Agency (EPA) Bayesian Space-time Downscaling Fusion Model (Downscaler) via EJScreen 2024. <https://www.epa.gov/hesc/rsig-related-downloadable-data-files#output> or <https://www.epa.gov/hesc/rsig-related-downloadable-data-files#output>

Particulate Matter 2.5 (PM_{2.5})



Ozone (O₃)

Definition: Maximum 8-hour average model predictions of concentrations of ground-level ozone in parts per billion (ppb).

Description: Ground-level ozone is an air pollutant that causes numerous adverse health effects, including respiratory irritation and exacerbation of lung disease. Ground-level ozone is formed from the reaction of oxygen-containing compounds with other air pollutants in the presence of sunlight. Ozone levels are typically at their highest in the afternoon and on hot days.

Indicator type: *Environmental Exposures*

Rationale: Of the six criteria air pollutants regulated under the Clean Air Act, ozone and particle matter pose the most widespread and significant health threats. Ozone is an extremely reactive form of oxygen. Ozone has the capacity to directly harm the respiratory tissues, producing oxidative stress and inflammatory reactions. Ozone exposure has been associated with several acute and chronic respiratory diseases, such as asthma, pneumonia, and respiratory mortality.ⁱⁱⁱ Over time, ozone can also produce a more systemic inflammatory process, which may contribute to heart disease and mortality, adverse pregnancy outcomes, and neurological diseases. In addition to its adverse impacts on human health, ozone can also cause harm to public welfare and the economy because it harms plant tissue and interferes with vegetation and crop growth.^{iv}

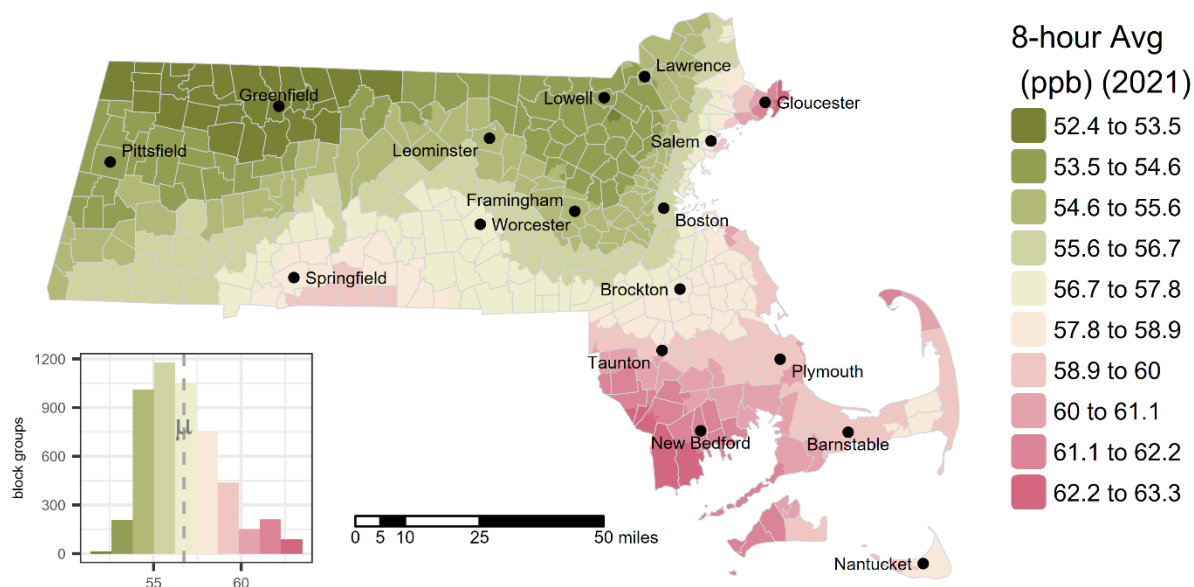
Method: State governments measured daily 8-hour maximum ozone concentration at multiple monitoring locations in parts per billion (ppb) and reported the monitored values to the U.S. Environmental Protection Agency (EPA). Air quality monitoring data from the State and Local Air Monitoring Stations (SLAMS) and numerical output from the Community Multiscale Air Quality (CMAQ) model were both input into the Bayesian Space-time Downscaling Fusion Model (Downscaler) to predict concentrations at the 2021 US census tract centroids encompassed by the CMAQ modeling domain. The mean values of all census tracts within a county were used to define the county Ozone measure. All census block groups received the same Ozone measure as the census tract in which they are contained.

Data years: 2021

Originating geographic scale of data: census tracts

Source: U.S. Environmental Protection Agency (EPA) Bayesian Space-time Downscaling Fusion Model (Downscaler) via EJScreen 2024. <https://www.epa.gov/hesc/rsig-related-downloadable-data-files#output> or <https://www.epa.gov/hesc/rsig-related-downloadable-data-files#output> or <https://screening-tools.com/epa-ejscreen>

Ozone (O₃)



Nitrogen dioxide (NO₂)

Definition: Average annual nitrogen dioxide (NO₂) levels expressed as part per billion (by volume). for 2020 at 1km grid resolution, aggregated to census block groups using mean pixel values.

Description: Nitrogen dioxide, or NO₂, is a gaseous air pollutant composed of nitrogen and oxygen and is one of a group of related gases called nitrogen oxides, or NO_x. NO₂ primarily gets in the air from the burning of fuel, such as gasoline, coal, oil, methane gas (natural gas), diesel, or wood. NO₂ forms from emissions from cars, trucks and buses, power plants, and off-road equipment. Breathing air with a high concentration of NO₂ can irritate airways in the human respiratory system and degrade ecosystems.^v

Indicator type: *Environmental Exposures*

Rationale: Nitrogen dioxide is one of six widespread air pollutants for which there are national air quality standards to limit their levels in the outdoor air. Breathing air with a high concentration of NO₂ can irritate airways in the human respiratory system. Such exposures over short periods can aggravate respiratory diseases, particularly asthma, leading to respiratory symptoms (such as coughing, wheezing or difficulty breathing), hospital admissions and visits to emergency rooms. Longer exposures to elevated concentrations of NO₂ may contribute to the development of asthma and potentially increase susceptibility

to respiratory infections. People with asthma, as well as children and the elderly are generally at greater risk for the health effects of NO₂. NO₂ along with other NO_x reacts with other chemicals in the air to form both particulate matter and ozone. NO₂ and other NO_x interact with water, oxygen and other chemicals in the atmosphere to form acid rain. Acid rain harms sensitive ecosystems such as lakes and forests. NO_x in the atmosphere contributes to nutrient pollution in coastal waters.^{vi}

Method: Source data for the NO₂ indicator are developed by NASA's Health and Air Quality Applied Sciences Team (HAQAST). The NO₂ indicator is developed from a regression model, satellite data, and large-scale models. The resulting product is an approximately one square kilometer grid of NO₂ concentrations. These grids are then projected to the 2020 Census block groups using a high-resolution NASA satellite product. The gridded data is then aggregated to census block group geographies using mean pixel values.

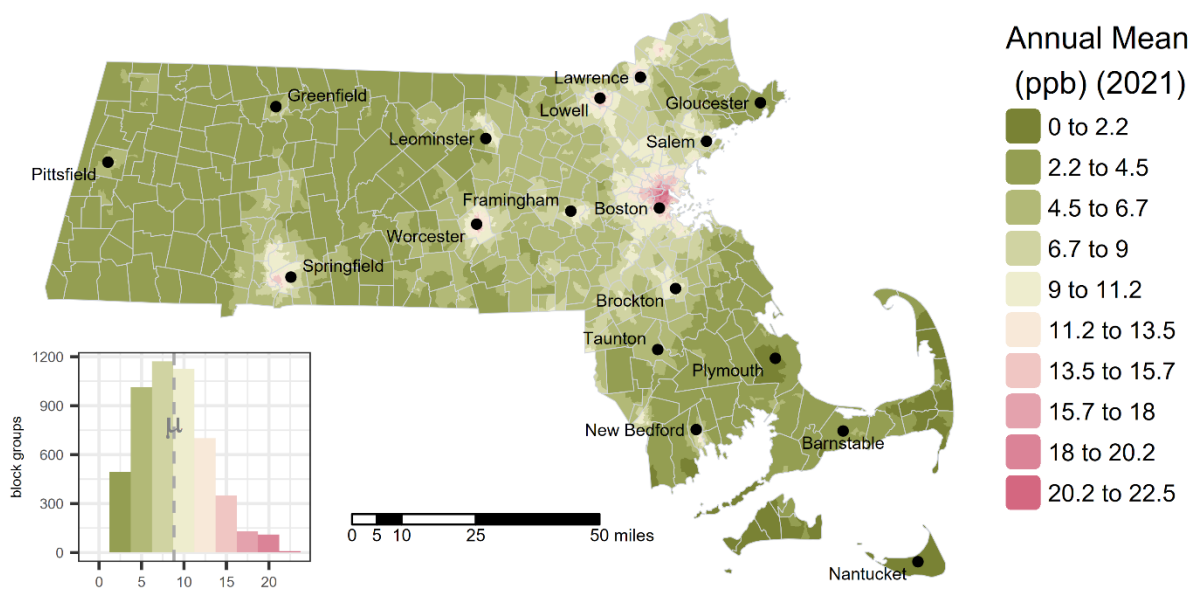
Data years: 2020

Originating geographic scale of data: 1km² pixels

Source: NASA's Health and Air Quality Applied Sciences Team (HAQAST) - Nitrogen Dioxide Surface-Level Annual Average Concentrations V1 via EJScreen 2024.

https://disc.gsfc.nasa.gov/datasets/SFC_NITROGEN_DIOXIDE_CONC_1/summary?keywords=TROPOMI%20NO2 or <https://screening-tools.com/epa-ejscreen>

Nitrogen Dioxide (NO₂)



Diesel Particulate Matter

Definition: Diesel particulate matter (PM) level in air measured in micrograms per cubic meter ($\mu\text{g}/\text{m}^3$).

Description: Diesel engines emit a mixture of chemicals called diesel particulate matter or diesel PM. Diesel PM can lead to asthma and respiratory illnesses and can worsen existing heart and lung disease. Diesel PM is highest near freeways, railyards, and other industrial sites where diesel engines are used.

Indicator type: *Environmental Exposures*

Rationale: Exhaust from trucks, buses, trains, and other equipment with diesel engines contain solid particles known as diesel particulate matter (diesel PM). Diesel PM contains hundreds of different chemicals.^{vii} Particulate matter, in general, has been linked to multiple adverse health outcomes, such as asthma, bronchitis, chronic obstructive pulmonary disease, kidney disease, diabetes, a variety of cancers, pregnancy outcomes, cardiovascular disease, pneumonia, and premature mortality, among others.^{viii} This measure is included in addition to traffic proximity and volume and the other air pollution indicators to reflect the unique exposure profile that diesel PM produces.

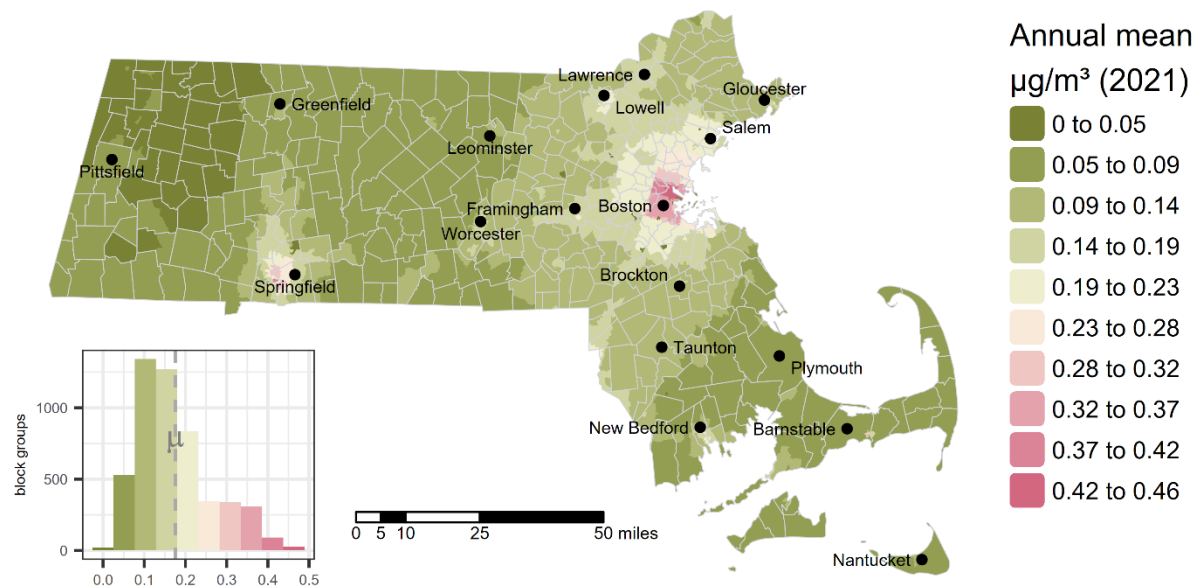
Method: The Diesel PM indicator is the estimated concentration of Diesel PM as provided by the 2020 AirToxScreen. The raw indicator is expressed in units of micrograms per cubic meter ($\mu\text{g}/\text{m}^3$), and reported at the Census block group-level. 2020 block groups are remapped to 2022 block groups.

Data years: 2020

Originating geographic scale of data: census block group

Source: U.S. Environmental Protection Agency (EPA) Air Toxics Screening Assessment via EJScreen 2024. <https://www.epa.gov/AirToxScreen> or <https://screening-tools.com/epa-ejscreen>

Diesel Particulate Matter



Drinking water Non-Compliance

Definition: Safe Drinking Water Act (SDWA) compliance performance score of a community water system (CWS) serving a census block group population.

Description: The goal of this indicator is to highlight populations served by community water systems (CWSs) that have challenges complying with National Primary Drinking Water Regulations (NPDWRs). The higher the Drinking Water Non-Compliance Indicator score, the more compliance challenges faced by the CWS(s) serving that census block group population. The drinking water non-compliance indicator distills hundreds of different violation types into a single value that attempts to capture Safe Drinking Water Act (SDWA) compliance performance of a CWS. The compliance data used by this indicator does not directly measure drinking water quality as this depends on violation type and other factors unique to each water system. The drinking water non-compliance indicator is only applicable to households that get their drinking water from a CWS, which are subject to National Primary Drinking Water Regulations (NPDWRs) under the SDWA. The indicator excludes households on private wells, for example, which are not regulated by the EPA. The indicator reflects compliance performance for the last 5-years (2019 Q4- 2023 Q4). Scores are only given to water systems for violations that have not been returned to compliance.^{ix}

Indicator type: *Environmental Exposures*

Rationale: Public water systems must meet health-based federal standards for contaminants, including performing regular monitoring and reporting. The Public Water System Supervision or PWSS program is designed to protect public health by ensuring the safety of drinking water. Public water systems are regulated by EPA, and primacy agencies (states, territories, and Tribes with EPA approval to implement the SDWA) and provide drinking water to 90% of the public. These public drinking water systems, which may be publicly- or privately-owned, serve at least 15 service connections or 25 persons.

Method: Violations are divided into 3 categories with associated point values weighted by severity (based on the three SDWA public notice violation tiers):

1. Health Based Violations (immediate impacts), 10 points:
 - a. Nitrate and/or Nitrite Maximum Contaminant Levels (MCLs), Acute Maximum Residual Disinfectant Level (MRDL), Revised Total Coliform Rule (R/TCR) Acute, Turbidity Treatment Technique (TT), Surface Water Treatment Rule Treatment Technique (SWTR TT).
2. Other Health Based Violations, 5 points:
 - a. Any other health-based violation (violation type MCL, MRDL or TT), plus Total Coliform Rule TCR Monitoring/Reporting (MR) repeat violations (violation code 25 and 26) and Nitrate MR.
3. Non-Health Based Violations, 1 point:
 - a. Any other Monitoring Violation (MON), Reporting Violation (RPT), Monitoring and Reporting (MR), or Other Violations categories.

For all 10- and 5-point violation types (see 1a. and 2a. above), the time (in years) a violation has been in non-compliance is added to the violation score. 1-point is added for every year of non-compliance, up to 5-years (5-points).

The violation data that makes up the drinking water non-compliance indicator was taken from EPA's internal Enforcement and Targeting Tool (ETT) which draws upon the SDWA system of record known as the Safe Drinking Water Information System, or SDWIS. SDWIS is updated on a quarterly basis and the violation data used to create the Drinking Water Non-Compliance Indicator score is from 2023 Q4.

The 5-year service area score of all violation types is first appended into utility service area boundaries:

$$\text{Service Area Score} = \sum (\text{Violation severity} + \text{Years in non-compliance}_{\text{health-based}})$$

The indicator score is then calculated at the block level and weighted by block population as a percent of its parent block groups' population (see equation below). If any part of the service area boundary is within a block, that block's score gets associated with that service area boundary(ies). If two or more service areas intersect the same block the scores are averaged. This method takes the most conservative approach: theoretically, if only one person is served by a public water system in that block, the score is applied to everyone living in that census block.

$$DWInScore_{blkGrp} = \sum \frac{(DWInScore_{blk})}{n} \times \% \text{ of block group population}$$

The block group score is the sum of the scores calculated at the associated census block level (see equation below). If only one system serves a block group, and the system encompasses all associated block group blocks, the block group score will be identical to the service area score for that system.

$$DWInScore_{blkGrp} = \sum (DWInScore_{blk})$$

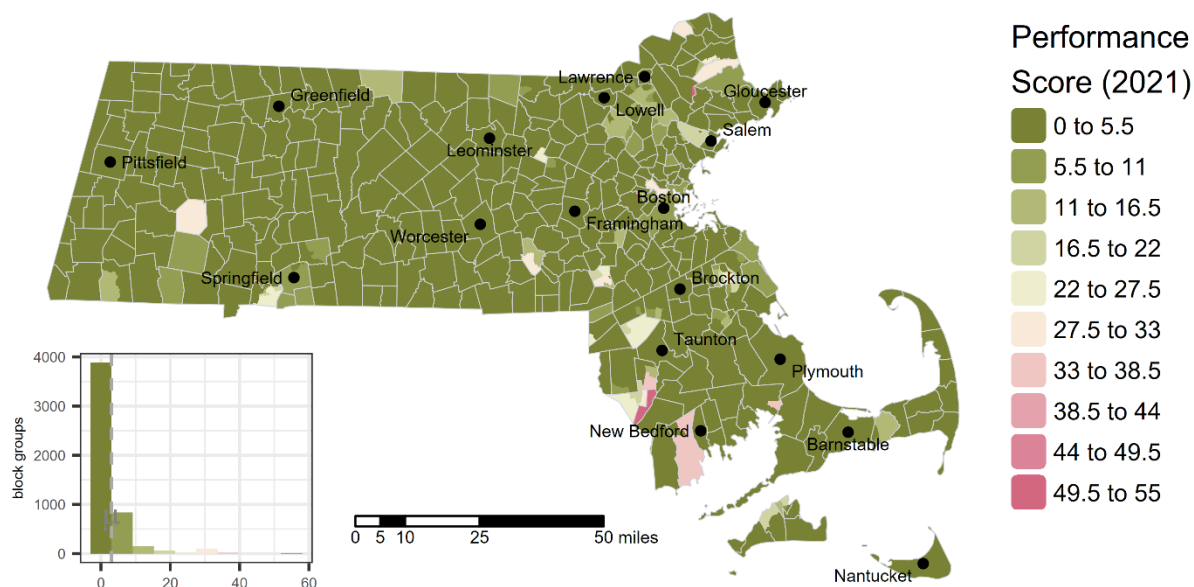
If no service areas intersect any part of a block group, that block group gets a null score. These areas are typically supplied by private well water.

Data years: 2019 - 2023

Originating geographic scale of data: census block groups

Source: EJScreen 2024 at <https://screening-tools.com/epa-ejscreen>

Drinking Water Non-Compliance



Air Toxics Cancer Risk

Definition: Risk of developing cancer due to inhalation exposure to air toxic compounds over a normal lifetime of 70 years, measured in incidents per million people.

Description: Toxic, or hazardous, air pollutants are substances that cause or are suspected of causing cancer, birth defects, or other serious harms. They can be gases, such as hydrogen chloride, benzene and toluene or compounds and metals such as asbestos, cadmium, mercury and chromium. Major sources of toxic air pollutants outdoors include emissions from coal-fired power plants, industries, and refineries, as well as from cars, trucks and buses. AirToxScreen provides a “snapshot” of outdoor air quality as it relates to air toxics. It also suggests the risks to human health if air toxic emission levels were to remain the same as those estimated for the assessment year. The estimates only reflect risks associated with chronic (assumed to be a 70 year lifetime) inhalation exposures to air toxics at the population level. The stressors evaluated through AirToxScreen can include any of the 188 current Hazardous Air Pollutants (HAPs) defined in the Clean Air Act and diesel PM.

Indicator type: *Environmental Exposures*

Rationale: Hazardous air pollutants, also known as toxic air pollutants or air toxics, are those pollutants that are known or suspected to cause cancer or other serious health effects, such as reproductive effects or birth defects, or adverse environmental effects. People exposed to toxic air pollutants at sufficient concentrations and durations may have an increased chance of getting cancer or experiencing other serious health effects.

Method: US EPA's AirToxScreen follows several basic steps to produce the final assessment:

- Compile National Emissions Inventory;
- Estimate ambient concentrations of air toxics across the U.S.;
- Estimate population exposures; and
- Characterize potential public health risks from inhalation.

The National Emissions Inventory (NEI) is a detailed, nationwide inventory of air toxics emissions. The NEI includes emissions from point, nonpoint and mobile sources, as well as emissions from biogenic sources and fires. These source data form the EPA's air emissions modeling platform and are the foundation of AirToxScreen's air quality modeling. NEI emissions and other needed data (e.g., meteorological data), are used as inputs to two air quality models used to estimate ambient air concentrations of air toxics: the American Meteorological Society/Environmental Protection Agency Regulatory Model (AERMOD) atmospheric dispersion model and the Community Multiscale Air Quality (CMAQ) photochemical model. AERMOD is used for all AirToxScreen air toxics modeled, and CMAQ is used for a list of 52 air toxics that are incorporated into CMAQ multipollutant version 5.4.

AirToxScreen uses emissions data compiled for a single year as inputs to air quality models. The models use these source data along with meteorological data for the same year to estimate ambient air concentrations of certain air toxics. EPA then combines these modeled concentrations with census data and other information to calculate exposure concentrations of the air toxics. AirToxScreen also estimates cancer risks and potential noncancer health effects associated with chronic inhalation exposure to the toxics.

Detailed description of the AirToxicScreen methodology is available at <https://www.epa.gov/AirToxScreen/2020-airtoxscreen-assessment-results>

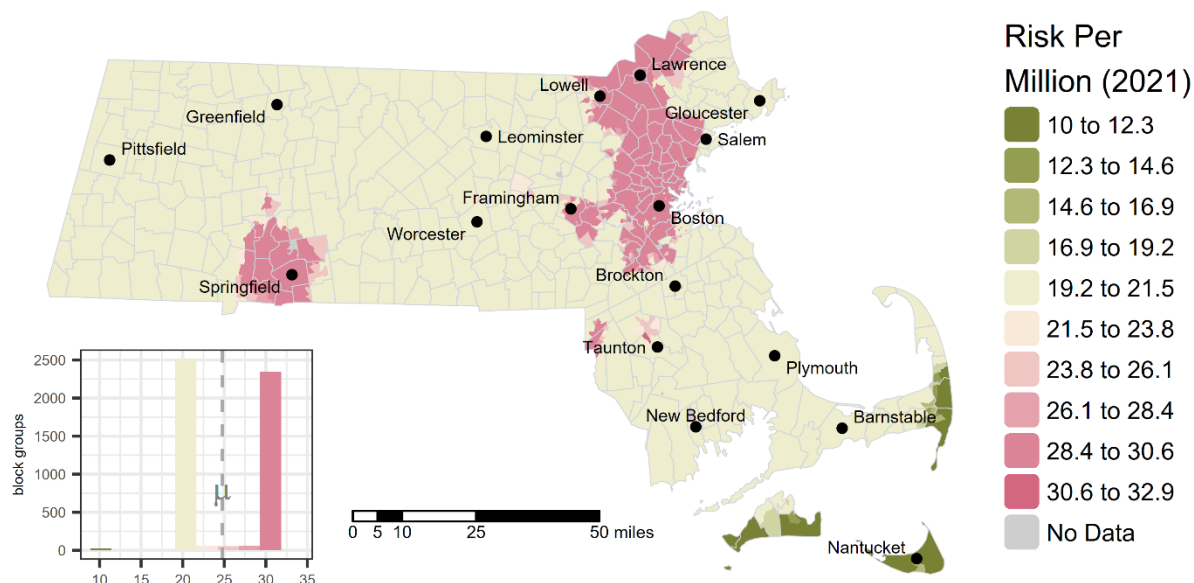
AirToxScreen Air Toxics Cancer Risk census block values were summed and averaged to obtain census block group values.

Data years: 2020

Originating geographic scale of data: census blocks

Source: US Environmental Protection Agency 2020 AirToxScreen: Assessment Results.
<https://www.epa.gov/AirToxScreen/2020-airtoxscreen-assessment-results>

Air Toxics Cancer Risk



Respiratory Hazard Index

Definition: Non-Cancer Respiratory Hazard Index; ratio of exposure concentration to a health-based reference concentration.

Description: The Respiratory Hazard Index represents the sum of hazard quotients for toxics that target the same organ or organ system with non-cancer effects. A hazard index of 1 or lower means air toxics are unlikely to cause adverse non-cancer health effects over a lifetime of exposure. Past assessment results suggest that the respiratory endpoint (the effect of air toxics on the lungs and the rest of the respiratory system) usually drives non-cancer health effects. Non-cancer hazards are presented in terms of the ratio between the exposure and a health-based reference concentration (RfC) for inhalation exposures (i.e., the HQ). Hazard indexes are summed across all air toxics modeled for the respiratory system.

Indicator type: *Environmental Exposures*

Rationale: Toxic, or hazardous, air pollutants are substances that cause or are suspected of causing cancer, birth defects, or other serious harms. They can be gases, such as hydrogen chloride, benzene and toluene or compounds and metals such as asbestos, cadmium, mercury and chromium. The U.S. Environmental Protection Agency has classified 188 pollutants as hazardous. Toxic air pollutants pose different risks to health depending on the specific pollutant, including: numerous forms of cancer, harm to the nervous system and brain, birth defects, irritation to the eyes, nose and throat, coughing and wheezing, impaired lung function, harm to the cardiovascular system, reduced fertility, and other effects.

Method: To evaluate the potential for noncancer adverse health effects, EPA uses chronic dose-response data to estimate a reference concentration (RfC) – the Exposure Concentration (EC) at which adverse health effects are assumed to be unlikely. EPA estimated chronic non-cancer hazards for AirToxScreen by dividing a chemical’s estimated long-term EC by the RfC for that chemical to yield a hazard quotient (HQ). An HQ value less than or equal to 1 indicates that the exposure is not likely to result in adverse noncancer effects. An HQ value greater than 1, however, does not necessarily suggest a likelihood of adverse health effects and cannot be interpreted to mean that adverse health effects are statistically likely to occur. The statement is simply whether, and by how much, an EC exceeds the RfC, indicating that a potential exists for adverse health effects. Chronic non-cancer hazards for multiple air toxics are estimated by summing chronic non-cancer HQs for individual air toxics that cause similar adverse health effects. The result is a hazard index (HI). Aggregation in this way produces a target-organ-specific HI, defined as a sum of HQs for individual air toxics that affect the same organ or organ system. The AirToxScreen modeling step generates ambient concentrations at the block level. For risk and exposure calculations, these results were aggregated from the block level to the tract level by taking a population-weighted average of all block-level concentrations within a given tract. Background concentrations, as well as estimated concentrations of secondary pollutants generated by the CMAQ model, were estimated for levels other than census tract and thus required interpolation “down” to the tract level. Background concentrations were estimated at the county level. To obtain tract-level concentrations, the county-level estimate was assigned to all census tracts within that county. For secondary pollutants, concentrations were estimated using CMAQ. The results for each grid were then applied evenly to all tracts located within the grid.

All census block groups received the same Respiratory Hazard Index measure as the census tract in which they are contained.

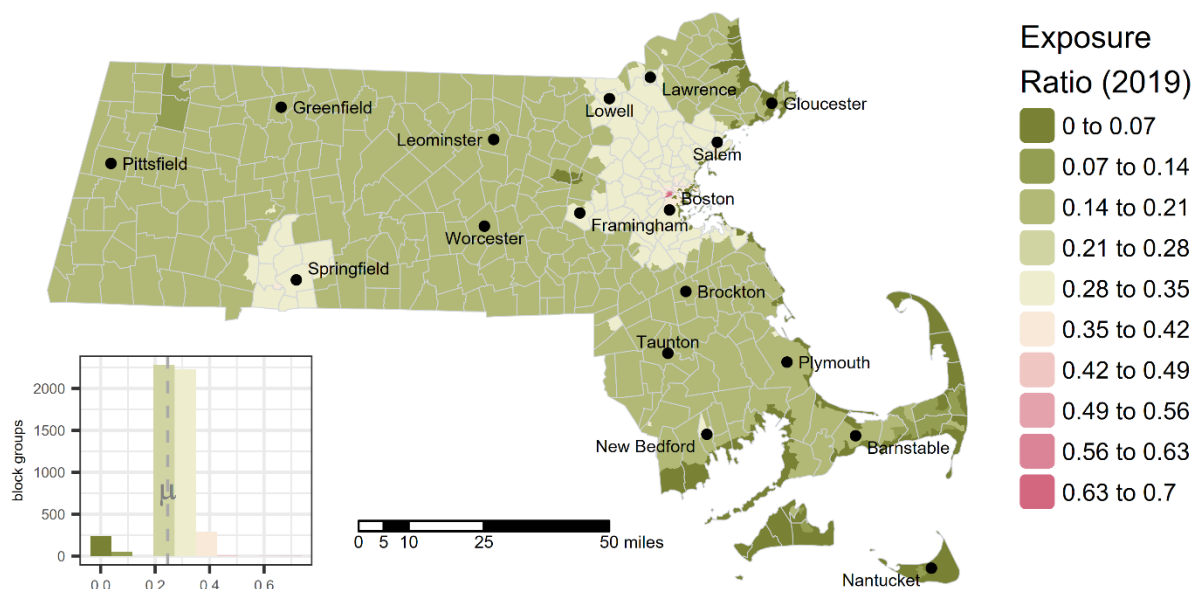
Data years: 2019

Originating geographic scale of data: census tracts

Source: US Environmental Protection Agency 2019 AirToxScreen: Assessment Results.

<https://www.epa.gov/AirToxScreen/2019-airtoxscreen-assessment-results>

Respiratory Hazard Index



Proximity to Heavy Traffic

Definition: Heavy traffic proximity impact index

Description: Heavy traffic proximity impact index is calculated as the sum of annual average daily traffic (AADT) at major roads within 10 kilometers of a census block centroid, divided by distance.

Indicator type: *Environmental Exposures*

Rationale: Traffic is a significant source of air pollution, particularly in urban areas, where more than 50% of particulate emissions come from traffic. Exhaust from vehicles contains a large number of toxic chemicals, including nitrogen oxides, carbon monoxide, and benzene. Traffic exhaust also plays a role in the formation of photochemical smog. Health effects of concern from these pollutants include heart and lung disease, cancer, and increased mortality.

Method: The traffic proximity indicator is based on AADT count divided by distance in meters from each census block centroid. The proximity score is based on the traffic within a search radius of 10 km. The closest traffic is given more weight, and the distant traffic is given less weight, through inverse distance weighting. The traffic proximity indicator values for each block centroid are averaged to the census block group in which they are contained.

Highway segments are from the HPMS lines and AADT counts are from the 2020 HPMS release, Federal Highway Administration (FHWA), U.S. Department of Transportation (DOT). Proximity scores were calculated by adding a 10km buffer to all non-zero population block groups; performing a one-to-many spatial join to identify all block group-segment pairs; assigning inverse distance scores for all identified 2020 Census blocks within each targeted block group with AADT multiplier (max score of 10); multiplying the scores by AADT and block population weights; and then aggregating the block scores to create a final block group score.

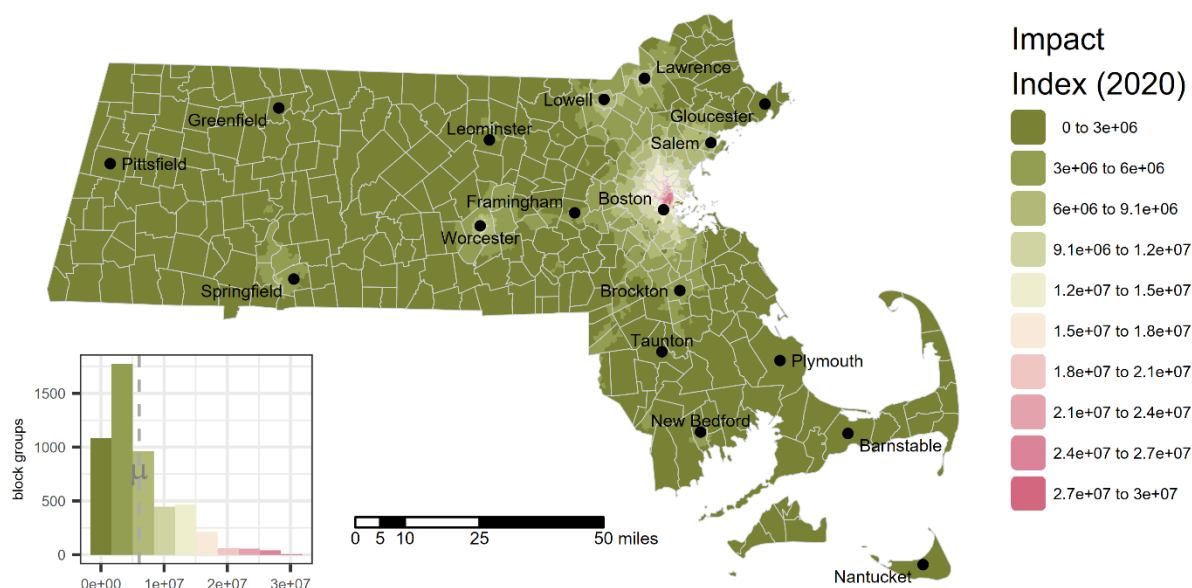
Data years: 2020

Originating geographic scale of data: census block groups

Indicator type: Environmental Exposures

Source: EJScreen 2024 at <https://screening-tools.com/epa-ejscreen>

Proximity to Heavy Traffic



Pollution and Climate Burden - Environmental Effects

Pollution Cleanup Sites

Definition: Weighted count of environmental cleanup sites requiring federal or state oversight for cleanup due to contamination.

Description: Sites undergoing cleanup actions by governmental authorities or by property owners have suffered environmental degradation due to the presence of hazardous substances. Of primary concern is the potential for people to come into contact with these substances. Some, such as “brownfield” sites, are also underutilized due to cleanup costs or concerns about liability. Since the nature and the magnitude of the threat and burden posed by hazardous substances vary among the different types of sites as well as the site status, the indicator takes both into account. Weights were also adjusted based on proximity to populated census blocks.

Indicator type: *Environmental Effects*

Rationale: Contaminated sites can pose a variety of risks to nearby residents. Hazardous substances can move off-site and impact surrounding communities through volatilization, groundwater plume migration, or windblown dust. Studies have found levels of organochlorine pesticides in blood^x and toxic metals in house dust,^{xi} as well as PFAS that were correlated with residents’ proximity to contaminated sites.^{xii}

Method: Sites were scored on a weighted scale of 0 to 12 in consideration of both the site risk and status. Higher weights were applied to Superfund and Chapter 21E sites where there was known groundwater contamination or an imminent hazard. Similarly, higher weights were applied to sites that are undergoing active remediation and oversight, relative to those in which no further action was required. The weights for all sites were adjusted based on the distance they fell from populated census blocks. Sites further than 1000m from any populated census block were excluded from the analysis. Site weights were adjusted by multiplying the weight by 1 for sites less than 250m, 0.5 for sites 250-500m, 0.25 for sites 500-750m, and 0.1 for sites 750-1000m from the nearest populated census blocks within a given census block group. Each census block group was scored based on the sum of the adjusted weights. Summed census block group scores were sorted and assigned percentiles based on their position in the distribution. The table below shows the weights for each site type.

Data years: National Priorities List and Superfund Alternative Approach Sites 2024; EPA ACRES brownfield sites 2024; MA DEP 21E sites 2025; MA DEP AUL sites 2025

Originating geographic scale of data: point locations

Weighting Matrix for Cleanup Sites

		Status		
		Low • Certified • Completed • No Further Action	Medium • Inactive – Needs Eval. • Certified Operation & Maintenance • Land Use Restrictions	High • Active • Inactive – Action Required
Risk	Low • No Significant Risk	0	4	6
	Medium • Not a substantial hazard but not “No Significant Risk” • Suspected Contamination	1	7	9
	High • Superfund • Groundwater contamination • Imminent hazard	2	10	12

Active: Identifies that an investigation and/or remediation is currently in progress and that DEP is actively involved, either in a lead or support capacity.

Certified Operation and Maintenance (O&M): Identifies sites that have certified cleanups in place but require ongoing O&M activities.

Certified: Identifies completed sites with previously confirmed releases that are subsequently certified by DEP as having been remediated satisfactorily under DEP oversight.

Inactive – Action Required: Identifies non-active sites where DEP has determined that a removal or remedial action or further extensive investigation is required.

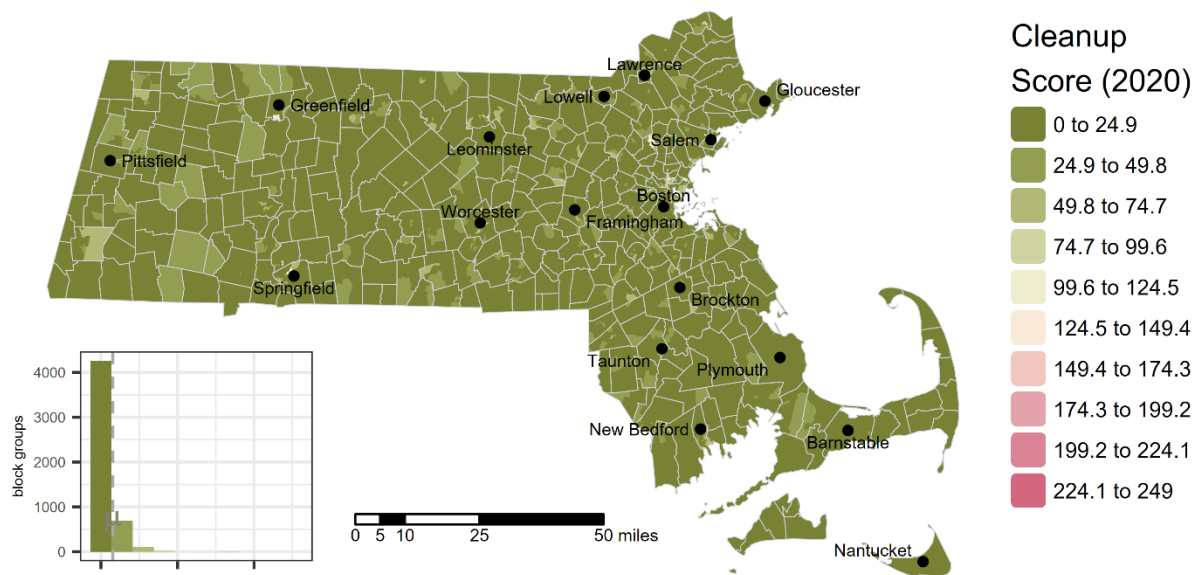
Inactive - Needs Evaluation: Identifies inactive sites where DEP has determined an evaluation is required.

No Further Action: Identifies completed sites where DEP determined that the property does not pose a problem to public health or the environment.

Superfund: Identifies sites where the US EPA proposed, listed, or delisted a site on the National Priorities List (NPL).

Sources: Superfund National Priorities List polygons from US EPA Office of Land and Emergency Management (OLEM) at https://edg.epa.gov/data/PUBLIC/OLEM/OLEM-OSRTI/NPL_Boundaries.zip; Brownfields sites from US EPA Assessment, Cleanup and Redevelopment Exchange System (ACRES) at <https://catalog.data.gov/dataset/acres-brownfields-properties>; MA DEP 21E sites from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-tier-classified-oil-and-or-hazardous-material-sites-mgl-c-21e>; MA DEP AUL sites from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-oil-and-or-hazardous-material-sites-with-activity-and-use-limitations-aul>;

Pollution Cleanup Sites



Groundwater Threats

Definition: Weighted count of groundwater threats.

Description: Many activities can pose threats to groundwater quality. These include the storage and disposal of hazardous materials on land and in underground storage tanks at various types of commercial, industrial, and military sites. Sites included here include Underground Storage Tanks (USTs) and facilities with groundwater discharge permits.

Indicator type: *Environmental Effects*

Rationale: The occurrence of storage tanks, leaking or not, provides a good indication of potential concentrated sources of some of the more prevalent compounds in groundwater. For example, the detection frequency of volatile organic compounds (VOCs) found in gasoline is associated with the number of UST sites within one kilometer of a well.^{xiii} The occurrence of chlorinated solvents in groundwater is also associated with the presence of cleanup sites.^{xiv} People who live near shallow groundwater plumes containing VOCs may also be exposed via the intrusion of vapors from soil into indoor air.^{xv}

Method: The weights for all sites were adjusted based on their distance from populated census blocks. Sites further than 1000m from any populated census block were excluded from the analysis. Site weights were adjusted by multiplying the weight by 1 for sites less than 250m, 0.5 for sites 250-500m, 0.25 for sites 500-750m, and 0.1 for sites 750-1000m from the nearest populated census blocks within a given block group. Sites outside of a census block group, but less than 1000m from one of that block group's populated blocks were similarly adjusted based on the distance to the nearest block from that block group. Each census block group was scored based on the sum of the adjusted weights for sites it contains or is near.

Data years: US EPA UST Finder 2018-2021; MA DEP Groundwater Discharge Permits 2025

Originating geographic scale of data: point locations

Sources: US EPA's UST Finder data is a national composite of leaking underground storage tanks, underground storage tank facilities, and underground storage tanks as of 2018-2021.

Data downloaded via ArcGIS Pro at

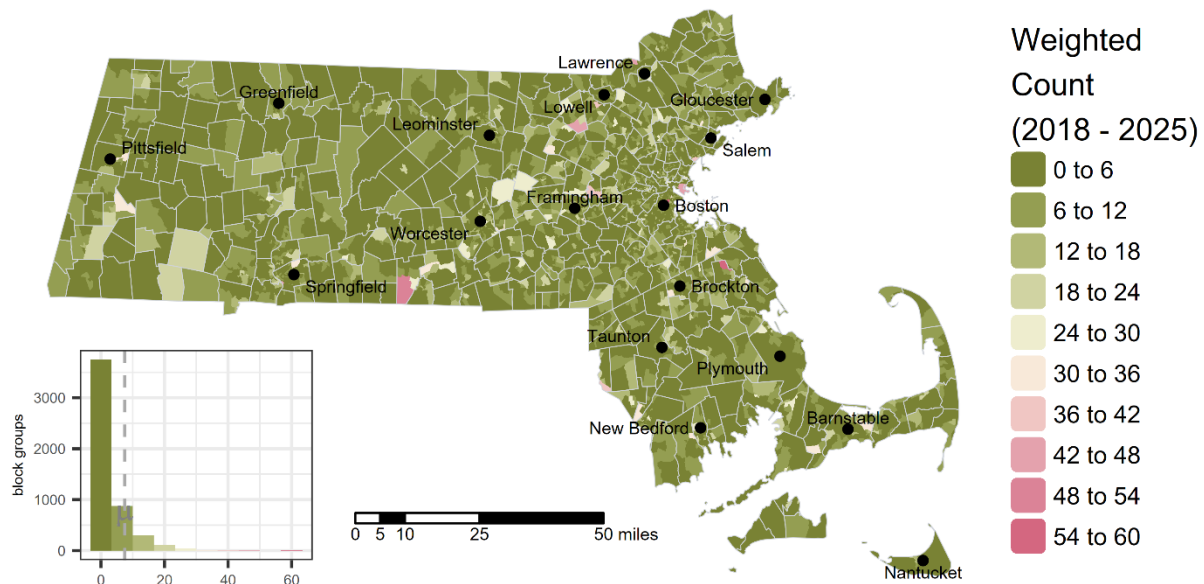
<https://epa.maps.arcgis.com/home/item.html?id=5a3ae0ed53564b6fa519f08e30e79e93>;

MA DEP Groundwater Discharge Permits from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-groundwater-discharge-permits>.

Weighting Matrix for Groundwater Threats

Site Type	Status/Type	Weight
Groundwater Discharge Permit Facilities	Industrial, Sanitary Discharge	5
	Car Wash, Laundromat, Reclaimed Water, Other	2
UST Sites	Open	3
	No Further Action	3
	Unknown	3

Groundwater Threats



Hazardous Waste Generators and Facilities

Definition: Weighted count of hazardous waste facilities, and hazardous waste generators within each census block group.

Description: Most hazardous waste must be transported from hazardous waste generators to permitted recycling, treatment, storage, or disposal facilities (TSDF) by registered hazardous waste transporters. Shipments are accompanied by a hazardous waste manifest. There are widespread concerns for both human health and the environment from sites that serve to process or dispose of hazardous waste. Many newer facilities are designed to prevent the contamination of air, water, and soil with hazardous materials, but even newer facilities may negatively affect perceptions of surrounding areas in ways that have economic, social and health impacts. The Massachusetts Department of Environmental Protection (MassDEP) Bureau of Air and Waste maintains data on permitted facilities that are involved in the treatment, storage, or disposal of hazardous waste as well as information on hazardous waste generators.

Indicator type: *Environmental Effects*

Rationale: Hazardous waste is, by definition, potentially dangerous or harmful to human health or the environment. The US Environmental Protection Agency and DEP both have

standards for determining when waste materials must be managed as hazardous waste. Hazardous waste can be liquids, solids, or contained gases. It can include manufacturing by-products and discarded used or unused materials such as cleaning fluids (solvents) or pesticides. Hexavalent chromium, a hazardous waste of particular human health concern, is generated as part of the chrome plating process.^{xvi} Used oil and contaminated soil generated from a site clean-up can be hazardous wastes. Generators of hazardous waste may treat waste onsite or send it elsewhere for disposal. The potential health effects that come from living near hazardous waste disposal sites have been examined in a number of studies.^{xvii} While there is sometimes limited assessment of exposures that occur in nearby populations, there are studies that have found health effects, including diabetes and cardiovascular disease, associated with living in proximity to hazardous waste sites.^{xviii}

Method: Permitted facility locations were obtained from the MassGIS data layer MassDEP Major Facilities. Facilities were scored on a weighted scale in consideration of the type, permit status, and compliance history for the facility (see Matrix). The weights for all facilities were adjusted based on the distance they fell from populated census blocks. All facilities further than 1,000m from any populated census block were excluded from the analysis. Site weights were adjusted by multiplying the weight by 1 for facilities less than 250m, 0.5 for sites 250-500m, 0.25 for sites 500-750m, and 0.1 for sites 750-1000m from the nearest populated census blocks within a given tract. Facilities outside of a census tract, but less than 1000m from one of that tract's populated blocks were similarly adjusted based on the distance to the nearest block from that tract. Each census block group was scored based on the sum of the adjusted weights for sites it contains or is near.

Data years: 2024

Originating geographic scale of data: point locations

Source: MassDEP Major Facilities from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-major-facilities>.

Weighting Matrix for Permitted Hazardous Waste Facilities and Hazardous Waste Generators

Permitted Hazardous Waste Facilities

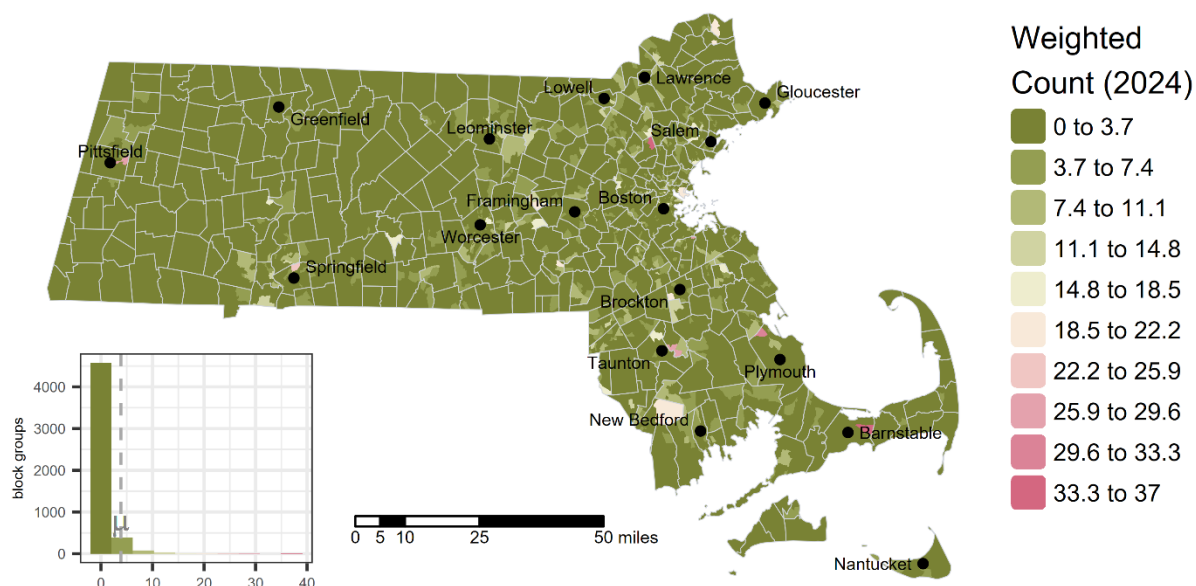
	Weight	Activity or Status
Facility Activity (base weight)	10	Hazardous Waste Treatment, Storage and/or Disposal Facility
	7	Hazardous Waste Recycler
Permit Type (additional weight)	1	Large facilities
	1	Air operating permit

	2	RCRA facilities
--	---	-----------------

Hazardous Waste Generators

Generator Type	Weight	Quantity of Waste
Large Quantity Hazardous Waste Generators or Large Quantity Toxic User	2	>1,000 tons/yr

Hazardous Waste Generators and Facilities



Solid Waste Sites and Facilities

Definition: Weighted count of solid waste sites and facilities.

Description: Many newer solid waste landfills are designed to prevent the contamination of air, water, and soil with hazardous materials. However, older sites that are out of compliance with current standards or illegal solid waste sites may degrade environmental conditions in the surrounding area and may expose nearby residents. Other types of facilities, such as composting, treatment and recycling facilities, may raise concerns about odors, vermin, and increased truck traffic.

Indicator type: Environmental Effects

Rationale: Solid waste sites can have multiple impacts on a community. Waste gases like methane and carbon dioxide can be released into the air from disposal sites for decades,

even after site closure.^{xix} Fires, although rare, can pose a health risk from exposure to smoke and ash.^{xx} Odors and the known presence of solid waste may impair a community's perceived desirability and affect the health and quality of life of nearby residents.^{xxi}

Method: Solid waste site locations were obtained from the MassGIS MassDEP Solid Waste Diversion and Disposal layer. The data includes locations of transfer, incineration and/or land disposal of solid waste in Massachusetts. The weights for all sites, including the large landfill perimeters, were adjusted based on the distance they fell from populated census blocks. Sites further than 1000m from any populated census block were excluded from the analysis. Site weights were adjusted by multiplying the weight by 1 for sites less than 250m, 0.5 for sites 250-500m, 0.25 for sites 500-750m, and 0.1 for sites 750-1000m from the nearest populated census blocks within a given census block group. Sites outside of a census block group, but less than 1000m from one of that tract's populated blocks were similarly adjusted based on the distance to the nearest populated block from that block group. Each census block group was scored based on the sum of the adjusted weights for sites it contains or is near. Summed census block group scores were sorted and assigned percentiles based on their position in the distribution.

Data years: 2025

Originating geographic scale of data: point locations

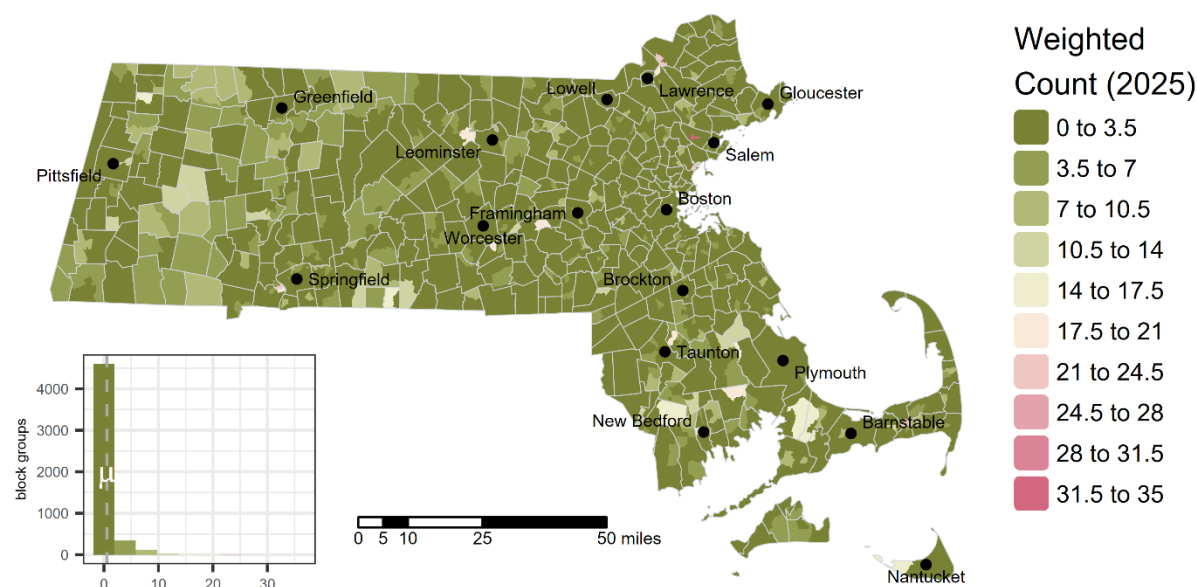
Source: MassDEP Solid Waste Diversion and Disposal layer from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-solid-waste-diversion-and-disposal>.

Weighting Matrix for Solid Waste Sites and Facilities

Category	Site of Facility Type
Waste combustion facility (active)	10
Solid Waste Landfill or Construction, Demolition and Inert (CDI) Debris Waste Disposal (active)	8
Unpermitted Dumping Grounds (inactive)	6
Transfer, processing, handling facility	5 Permitted large volume, C&D 2 Permitted small volume, organic or wood waste
Waste Tire, recycling, wood waste	4
Composting	4 Site assigned composting facility 2 Site assignment exempt composting facility
Aerobic/Anaerobic Digesting	3

Solid Waste Disposal Site, historic incinerator or combustion facility (closed, inactive)	1

Solid Waste Sites and Facilities



Impaired Water Bodies

Definition: Count of pollutants across all water bodies designated as impaired within the area.

Description: Contamination of streams, rivers, lakes, and coastal waters by pollutants can compromise the use of the water body for drinking, swimming, fishing, aquatic life protection, and other beneficial uses. When this occurs, such water bodies are considered “impaired.” Information on impairments to these water bodies can help determine the extent of environmental degradation within an area.

Indicator type: *Environmental Effects*

Rationale: Rivers, lakes, ponds, estuaries and marine waters in Massachusetts are important for many different uses. Water bodies used for recreation may also be important to the quality of life of nearby residents if subsistence fishing is critical to their livelihood. Water bodies also support abundant flora and fauna. Alterations in natural conditions in

aquatic environments can affect biological diversity and overall health of ecosystems. Aquatic species important to local economies may be impacted if the habitats where they seek food and reproduce are changed. Marine wildlife like fish and shellfish that are exposed to toxic substances may potentially expose local consumers to toxic substances as well. Excessive hardness, unpleasant odor or taste, turbidity, color, weeds, and trash in the waters are types of pollutants affecting water aesthetics, which in turn can affect nearby communities.

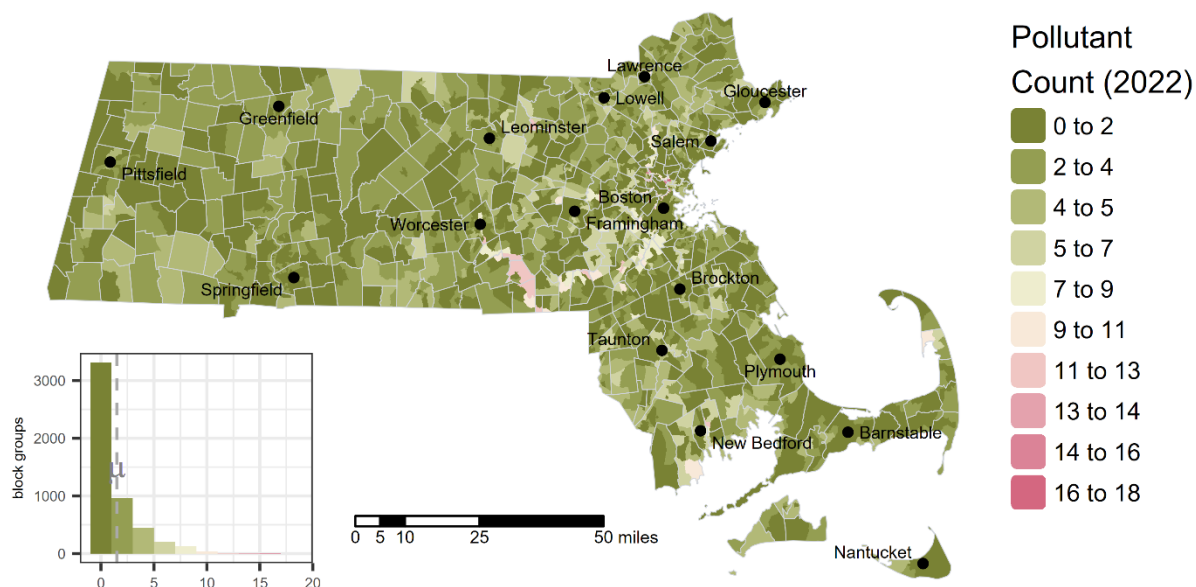
Method: The number of pollutants listed in streams or rivers that fell within 1 kilometer (km) or 2 km respectively of a census block group's populated blocks were counted. The 2 km buffer distance was applied to major rivers (>100 km in length). The 1 km buffer distance was applied to all smaller streams/rivers. The number of pollutants listed in lakes, bays, estuaries or shoreline that fell within 1 km or 2 km of a census block group's populated blocks were counted. The 2 km buffer distance was applied to major lakes or bays greater than 25 square kilometers in size. The 1 km buffer distance was applied for all other lakes/bays. The two pollutant counts were summed for every census block group. Each census block group was scored based on the sum of the number of individual pollutants found within and/or bordering it. For example, if two stream sections within a census block group were both listed for the same pollutant, the pollutant was only counted once. Summed census block group scores were sorted and assigned percentiles based on their position in the distribution.

Data years: 2022

Originating geographic scale of data: water body linear and polygon features

Source: MassDEP 2022 Integrated List of Waters (305(b)/303(d)) from MassGIS at <https://www.mass.gov/info-details/massgis-data-massdep-2022-integrated-list-of-waters-305b303d>.

Impaired Water Bodies



Pollution and Climate Burden - Climate Risks

Drought

Definition: Sum of weekly total percent of an area experiencing a severe, extreme, or exceptional drought (categories D2, D3, or D4), adapted from Colorado EnviroScreen.

Description: A drought is a period of time when an area experiences less rainfall or snowfall than normal. Climate change increases the frequency and severity of drought. This indicator represents how much of a county was affected by drought over time. These levels of drought imply some level of voluntary or mandated water use restrictions and observable damage or loss of pasture and crops. The U.S. Drought Monitor reports the percentage of each county experiencing each of the six drought levels (None, D0, D1, D2, D3, and D4) each week.

Indicator type: Climate Risks

Rationale: Drought is an important climate health indicator because it reflects areas with an increased risk of wildfires, dust storms, shortage of local water and food sources, poor quality drinking water, and sanitation and hygiene restrictions. Drought can lead to dehydration, stress, anxiety, depression, lung and heart diseases, stroke, and infectious diseases, among others.^{xxii} Severe, extreme, and exceptional drought categories are more

likely to impact populations through water shortages, water restrictions, or crop or pasture losses.

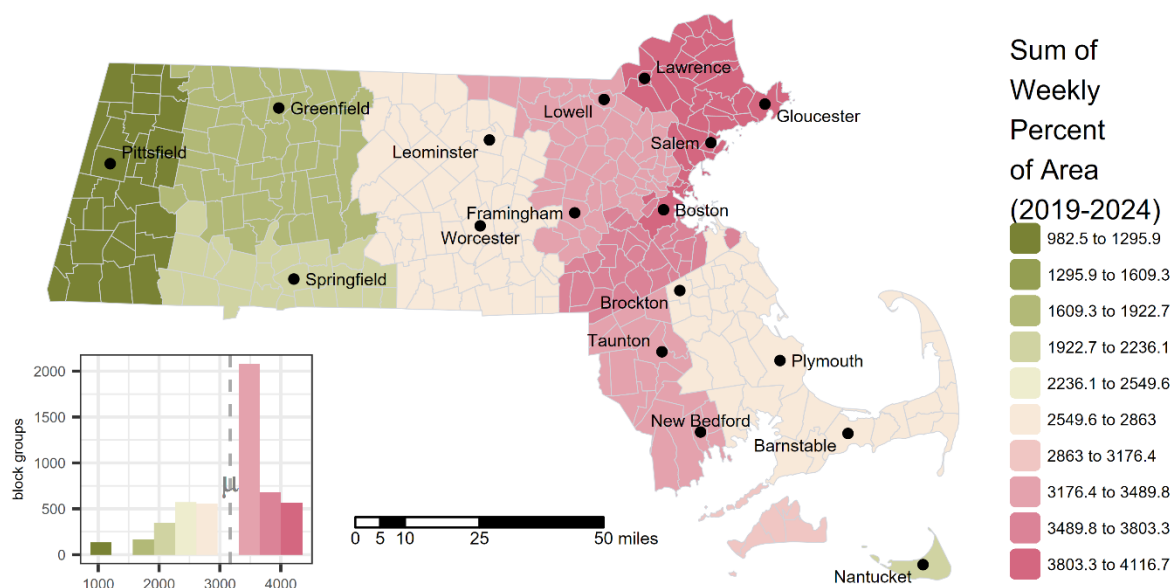
Method: The U.S. Drought Monitor reports the percentage of each county experiencing each of the six drought levels (None, D0, D1, D2, D3, and D4) each week. The sum of areas experiencing D2, D3, D4 level droughts was calculated weekly across all weeks from January 2019 to December 2024. The sum of the weekly drought values across that time period was used to define the Drought measure. All census block groups received the Drought value for the county in which they are located.

Data years: 2019 - 2024

Originating geographic scale of data: county

Source: U.S. Drought Monitor 2019-2024 <https://droughtmonitor.unl.edu/Data.aspx>

Drought



Wildfire Risk

Definition: Mean wildfire hazard potential.

Description: A wildfire is an uncontrolled fire in nature, such as in forests or grasslands. Wildfires affect people's physical, mental, and financial health. Wildfires threaten

ecosystems and water supplies and produce harmful smoke, which can trigger health problems like asthma or stroke. Climate change increases the risk and severity of wildfires.

Indicator type: Climate Risks

Rationale: Wildfires can lead to direct injuries and death among those living in affected areas.^{xxiii} Other health risks can be associated with the evacuation of patients, reduced access to health services, and the impact on the quality and availability of local water resources. Wildfires also produce smoke that impacts communities in the affected areas. The smoke can travel far distances, even reaching communities in other states. Wildfire smoke has been associated with multiple health outcomes, such as adverse pregnancy outcomes, impacts to children's health (e.g., asthma), and impacts to adults (e.g., cardiovascular and respiratory diseases). Exposure to wildfire smoke can ultimately increase the risk of premature mortality.^{xxiv}

Method: The mean wildfire hazard potential within each geographic area is used as the Wildfire risk score. Wildfire Hazard Potential (WHP) is provided as a 270-meter resolution raster. The mean WHP of all pixels or raster grid cells falling within each census block group were calculated.

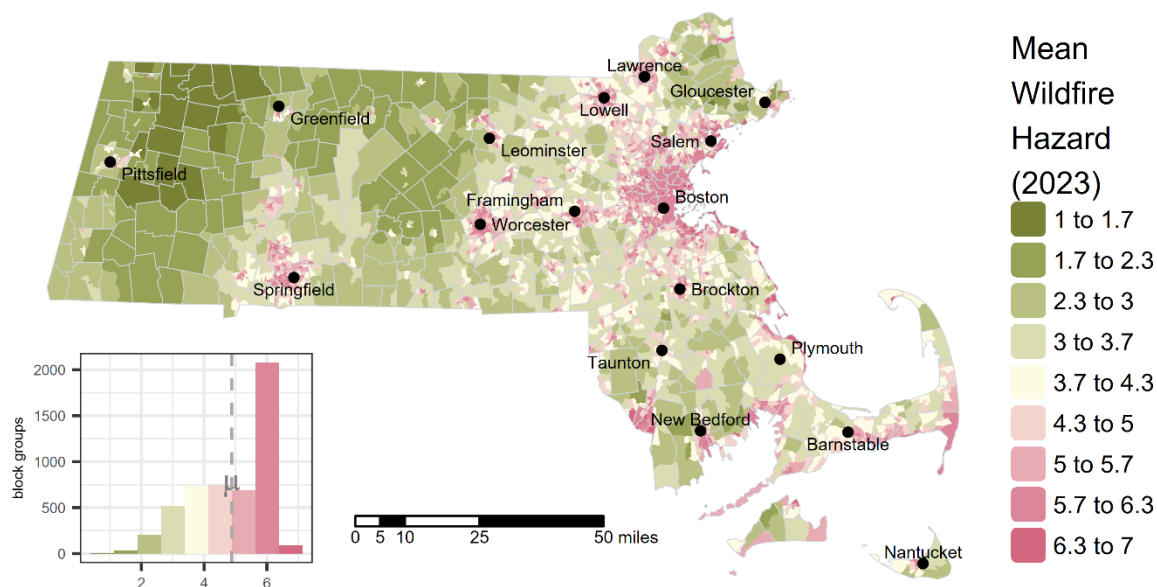
Data years: 2023

Originating geographic scale of data: 270-meter pixels

Source: U.S. Department of Agriculture (USDA), U.S. Forest Service (USFS)

<https://www.fs.usda.gov/rds/archive/catalog/RDS-2015-0047-4>

Wildfire Risk



Flood Risk

Definition: Percentage of each geographic area where there is at least a one percent chance of flooding annually.

Description: Special Flood Hazard Areas (SFHAs) are defined by the Federal Emergency Management Agency as the area that will be inundated by the flood event having a 1-percent chance of being equaled or exceeded in any given year.^{xxv} The 1-percent annual chance flood is also referred to as the base flood or 100-year flood and is considered to be an area with a high risk of flooding.^{xxvi} Flooding can affect people's physical, mental, and financial health. Climate change increases the risk and severity of floods.

Indicator type: Climate Risks

Rationale: Floods are the most common form of weather-related disaster in the US, and they are the second-leading cause of weather-related fatalities.^{xxvii} Floods can immediately impact health due to drowning, injuries, and hypothermia. Other health risks can be associated with the evacuation of patients, reduced access to health services, or the impact on basic services, including drinking water supply and quality. In the medium and long term, floods have been associated with infectious diseases (due to poor quality drinking water, lack of hygiene and sanitation, infected wounds, or insect bites), poisoning

(due to chemicals in floodwater), and mental health issues (e.g., stress, anxiety, depression). Flood exposure can ultimately increase the risk of premature mortality.^{xxviii}

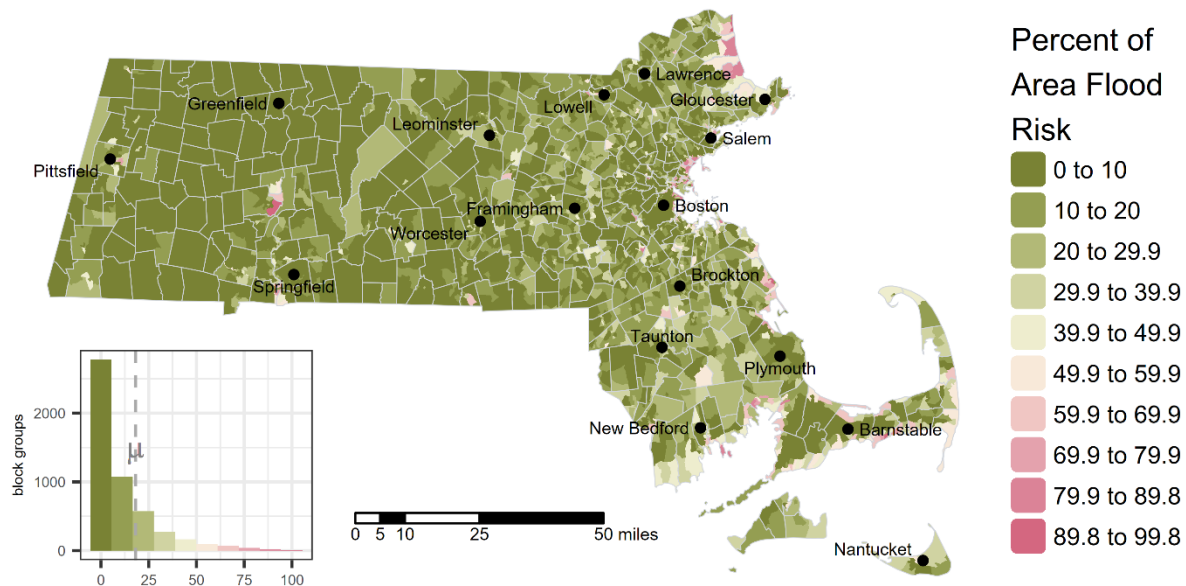
Method: FEMA National Flood Hazard Layer (NFHL) digital Flood Insurance Rate Maps (DFIRMs) were downloaded from MassGIS. However, as of July 2025, NFHL DFIRMs are not available for the northwest quadrant of Massachusetts – Berkshire County, Hampshire County, Franklin County, and portions of northern Worcester County. These areas were supplemented with data from the FEMA Q3 Flood Zones from Paper FIRMs layer from MassGIS, which cover most of the northwest except for Franklin County. Flood risk data for Franklin County was acquired from FEMA’s National Risk Index – “riverine flooding - exposure - impacted area” layer, which is the percentage of area exposed to flood risk, and is summarized by census tract. For NFHL DFIRMs and FEMA Q3 layers, the area of all features within the 1% Annual Chance Flood Hazard within each census block group was divided by the total area of the census block group. For NFHL DFIRMs, these included areas within zones "A", "AE", "AH", "AO", and "VE". For FEMA Q3 data, these included areas within zones "AE", "A", "D", and "AO". If no flood areas were found within the census block group, a value of zero was used. For FEMA NRI data in Franklin County, all census block groups were assigned the value of the census tract in which they are contained.

Data years: FEMA NFHL data 2023; FEMA Q3 Flood Zones from Paper FIRMs data 1997 - 2013; FEMA National Risk Index data for Franklin County 2025

Originating geographic scale of data: delineated flood polygons for NFHL and Q3 data; census tracts for Franklin County

Source: FEMA National Flood Hazard Layer (NFHL) from MassGIS at <https://www.mass.gov/info-details/massgis-data-fema-national-flood-hazard-layer>; FEMA Q3 Flood Zones from Paper FIRMs from MassGIS at <https://www.mass.gov/info-details/massgis-data-fema-q3-flood-zones-from-paper-firms>; FEMA’s National Risk Index – “riverine flooding - exposure - impacted area” layer from <https://hazards.fema.gov/nri/data-resources>

Flood Risk



Extreme Heat Days

Definition: Average number of days between May and September from 2019 through 2023 in which daily high temperature exceeded the 90th percentile of historical daily high temperatures.

Description: Extremely hot days can cause health risks due to heat exhaustion and dehydration. Extremely hot days can also cause electrical outages, which can hurt the health of people who use machines for healthcare purposes. Climate change increases the number and severity of extreme heat days. This indicator describes the estimated annual average number of days from May to September that were much hotter than usual (a temperature above the 90th percentile of temperatures from 1979 to 2019).

Indicator type: Climate Risks

Rationale: Heat-related illnesses occur when people are exposed to an abnormal or prolonged amount of heat without relief or adequate fluid intake.^{xxix} Heat-related illnesses include heat stroke, exhaustion, cramps, rash, and sunburn. Heat-related illnesses can also increase the risk of premature mortality. Infants, children, older adults, and those living with medical conditions are more susceptible to developing health-related issues.

Moreover, outdoor workers, athletes, and those without access to shelter, air conditioning, and drinking water are also more vulnerable to extreme heat days.^{xxx}

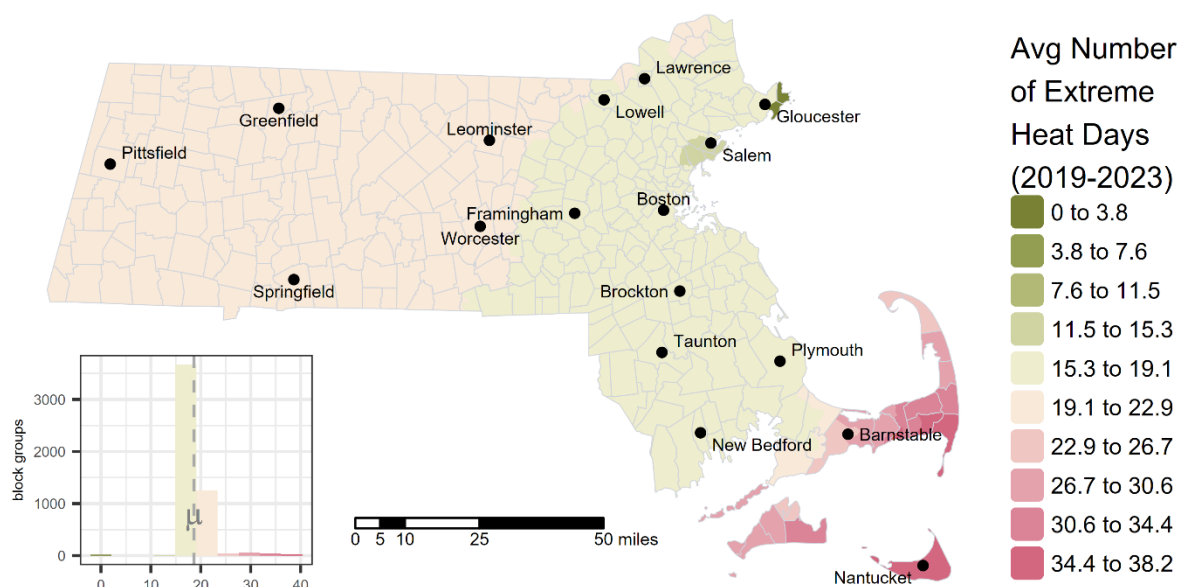
Method: Data on the number of the extreme heat days was downloaded from the National Environmental Public Health Tracking Network tool using the following query: Heat & Health-Related Illness (HRI > Historical Temperature & Heat Index > Annual Number of Extreme Heat Days > MA Census Tracts > 2019 - 2023 > Heat Metric Max Daily Temp > Relative Threshold 90th Percentile. All census block groups received the Extreme heat days value for the census tract in which they are located.

Data years: 2019 – 2023

Originating geographic scale of data: census tracts

Source: National Environmental Public Health Tracking Network via the U.S. Centers for Disease Control (CDC), Heat & Heat Related Illness (HRI), Historical Temperature & Heat Index, 2019-2023 <https://ephtracking.cdc.gov/>

Extreme Heat Days



Population Characteristics - Sensitive populations

Pediatric Asthma

Definition: Population-weighted average asthma prevalence (percentage of K-8 enrollment).

Description: Asthma prevalence in schools is defined as the percentage of enrolled students reported by school nurses to have asthma during a school year. Schools are public, charter, and private. School-based asthma data are reported annually by school nurses through the Massachusetts Department of Public Health Pediatric Asthma and Diabetes Survey for students in grades K-8 who have ever been diagnosed with asthma. Average prevalence for each census block group was calculated based on the average prevalence of all schools within ½ mile, weighted by school enrollment.

Indicator type: *Sensitive Populations*

Rationale: Asthma is an illness that affects the respiratory tract and airways that carry oxygen into and out of the lungs. During an asthma attack, these airways constrict, resulting in wheezing and difficulty breathing. Asthma can affect people of all ages. However, it often starts in childhood and is more common in children than adults. Asthma is a common chronic disease that continues to increase in prevalence. It is the most common chronic disease in children. The state of Massachusetts has an elevated rate of asthma compared to the national prevalence rate.^{xxxix} Causes of asthma are unknown. However, episodes of asthma (asthma attacks) can be triggered by certain environmental pollutants such as air pollution, mold, pets/pet dander, and dust mites. A number of studies have reported links between exposure to air pollution and asthma. Several criteria air pollutants are known to exacerbate asthma symptoms and contribute to asthma development. In addition, climate change may affect air quality and worsen asthma in several ways. Climate-driven changes in weather can increase certain criteria air pollutants like ozone and particulate matter, as well as other triggers such as pollen, mold, dust mites, and bacteria, all of which may exacerbate asthma. Climate change also leads to more frequent wildfires, which worsen respiratory illnesses including asthma. Reducing exposure to these pollutants can help prevent symptoms. Asthma impacts certain groups more than others, including children, older adults, people of color, and those of lower socioeconomic status. These disparate impacts may be due to environmental factors related to socioeconomic status, like housing conditions or proximity to sources of pollution. Children of color have a higher prevalence of asthma and asthma-related emergencies. Children with asthma seem to be impacted by particle pollution more so than adults with asthma.^{xxxii}

Method: MDPH calculated the school rates for asthma prevalence and provided these to MassDEP with values per school. Each school was associated with census block groups within ½ mile. Average prevalence for each census block group was calculated based on

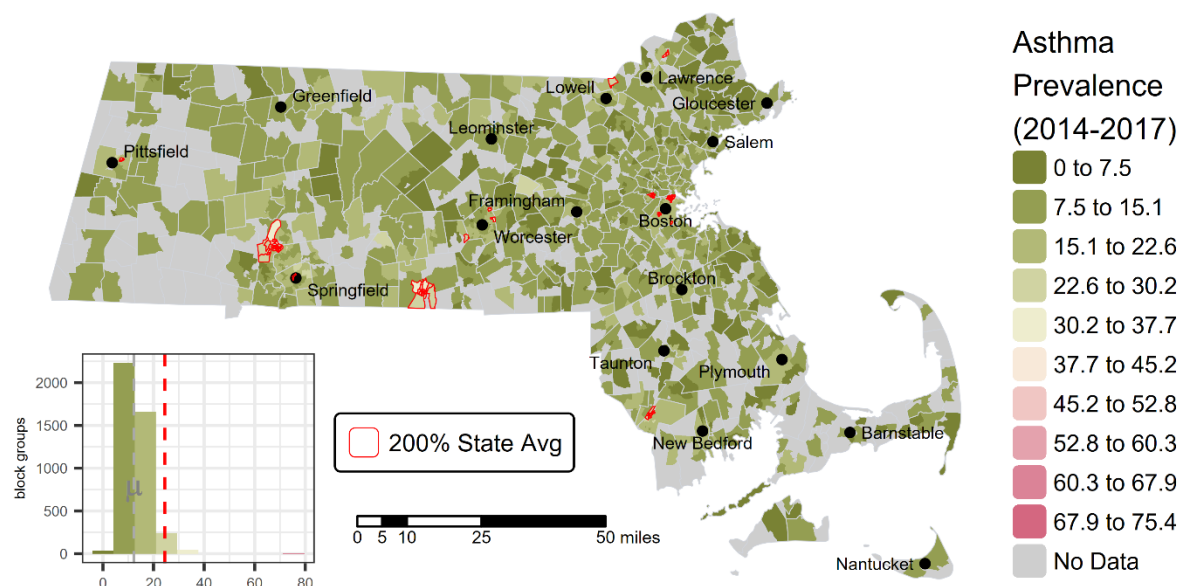
the average prevalence of all schools within ½ mile, weighted by school enrollment. Schools missing asthma or enrollment data were excluded.

Data years: 2014 – 2017

Originating geographic scale of data: school point locations

Source: MassDEP Cumulative Impact Analysis in Air Quality Permitting at <https://www.mass.gov/info-details/cumulative-impact-analysis-in-air-quality-permitting#cia-guidance-and-tools->

Pediatric Asthma Prevalence



Elevated Blood Lead Levels in Children

Definition: 5-year average prevalence of elevated (≥ 5 $\mu\text{g}/\text{dL}$ estimated confirmed) childhood blood lead levels in children (ages 9-47 months).

Description: Exposure to lead through paint is the most significant source of lead exposure for children. Lead is a toxic heavy metal and occurs naturally in the environment. However, most of the high levels of lead found in our environment result from human activities. Historically, lead was used as an additive in gasoline and as a primary ingredient in house paint. Lead levels in the United States have declined over the past five decades due to

various regulations. However, lead still persists in older buildings containing lead paint, as well as old plumbing and contaminated soil. Young children absorb lead more easily than adults. The Massachusetts Lead Poisoning Prevention and Control Act is a state law that requires all children to be screened each year for lead poisoning through age three. Children living in high-risk communities must be screened each year through age four. All children must show proof of screening at least once to enter daycare, pre-kindergarten programs, and kindergarten.

Indicator type: Sensitive Populations

Rationale: Young children are especially susceptible to the effects of lead exposure and can suffer profound and permanent adverse health effects, particularly in the brain and nervous system.^{xxxiii} This increased susceptibility is due to their unique exposure pathways (e.g., dust-to-hand-to-mouth), developing brains, and differences in the absorption of ingested lead.^{xxxiv} Lead in the body can: hurt the brain, kidneys, and nervous system, slow down growth and development, make it hard to learn, damage hearing and speech, and cause behavior problems. There are no known safe levels of lead exposure, and levels that were previously considered safe are now known to cause subtle, chronic health effects. In October 2021, the Centers for Disease Control and Prevention (CDC) lowered the blood lead reference value (BLRV) from 5 micrograms per deciliter (µg/dL) to 3.5 µg/dL.

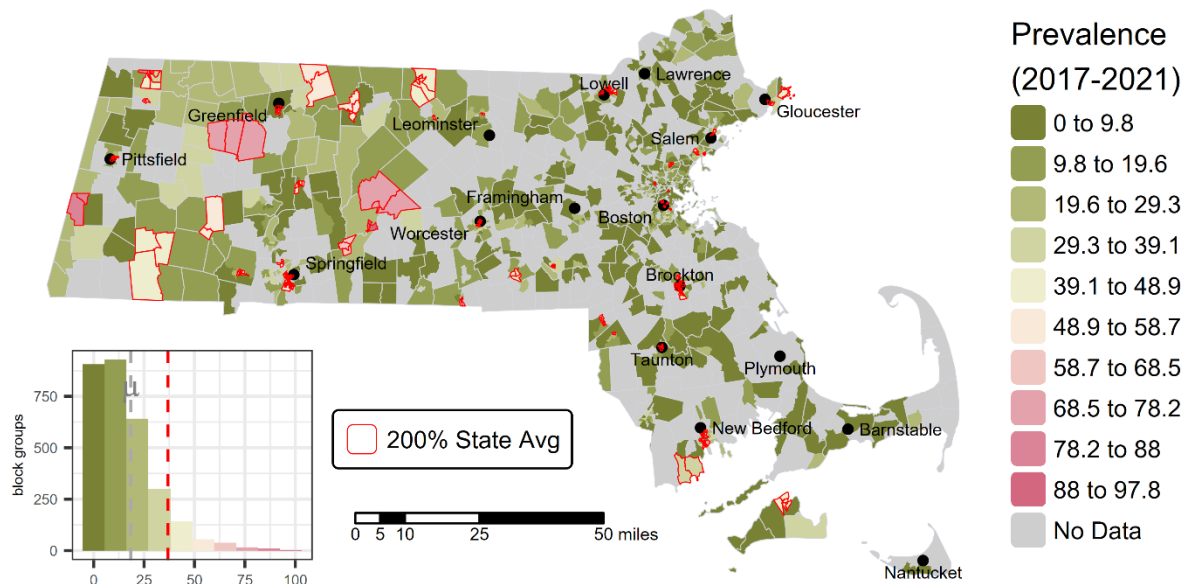
Method: All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2017 – 2021

Originating geographic scale of data: census tracts

Source: Massachusetts Environmental Public Health Tracking via MassDEP Cumulative Impact Analysis in Air Quality Permitting at <https://www.mass.gov/info-details/cumulative-impact-analysis-in-air-quality-permitting#cia-guidance-and-tools->

Elevated Blood Lead Levels



Low Birth Weight Infants

Definition: 5-Year annual average low birth weight (less than 2,500 grams) per 100 full-term births.

Description: Birth weight, which is impacted by the length of gestation and fetal growth, is considered low when infants weigh less than 2,500 grams (5lbs and 8 ounces). Preterm birth and low birth weight are associated with a range of health issues for children, including breathing problems, feeding difficulties, developmental delay, hearing and vision problems, and more.

Indicator type: Sensitive Populations

Rationale: Compared to normal birth weight infants, low birth weight infants may be more at risk for illness through the first six days of life, infections, and long-term impairment, such as delayed motor and social development or learning disabilities. ounces. Preterm birth and low birth weight are associated with a range of health issues for children, including breathing problems, feeding difficulties, developmental delay, hearing and vision problems, and more.^{xxxv} There are many potential causes for adverse birth outcomes, including maternal health conditions and lifestyle factors.^{xxxvi} The role that exposure to environmental contaminants plays in adverse birth outcomes is not yet fully understood,

particularly in the potential causation of preterm birth and low birth weight.^{xxxvii} Various exposures have been implicated as risk factors for full-term low birth weight, including maternal exposure to lead and exposure to toxic substances in air, water or food.^{xxxviii} Research suggests that exposure to PM2.5, carbon monoxide, and nitrogen oxides might contribute to preterm birth and low birth weight.^{xxxix} Exposure to lead is also known to cause reduced fetal growth and may cause preterm birth.^{xl} Preterm and low birth weight infants have a higher risk of health issues and mortality from childhood to adulthood.^{xli}

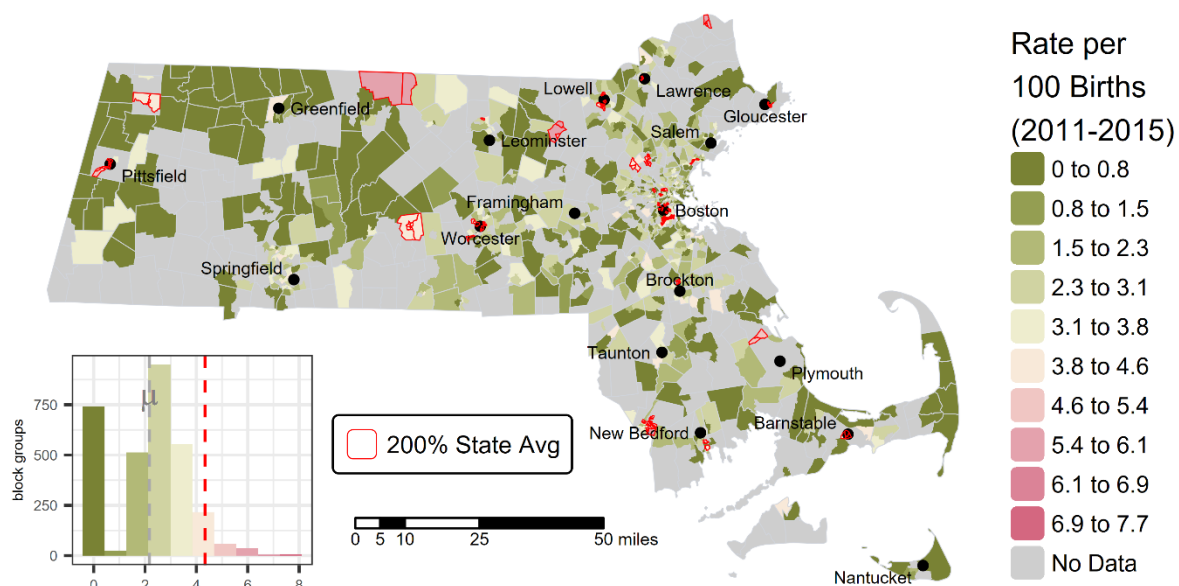
Method: All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2011-2015

Originating geographic scale of data: census tract

Source: Massachusetts Department of Public Health Birth Outcomes Data of Massachusetts Residents at <https://www.mass.gov/info-details/birth-outcomes-data-of-massachusetts-residents> via MassDEP Cumulative Impact Analysis in Air Quality Permitting at <https://www.mass.gov/info-details/cumulative-impact-analysis-in-air-quality-permitting#cia-guidance-and-tools->

Low Birth Weight Infants



Premature Mortality (PM)

Definition: Age-adjusted premature mortality rate (per 100,000).

Description: Premature mortality tracks unfulfilled life expectancy and is based on deaths prior to age 75. Premature mortality includes unintentional injuries such as motor vehicle-related deaths, poisonings, falls, fires, and drownings that were not intended to occur.

Deaths are for all age groups.

Indicator type: Sensitive Populations

Rationale: Premature mortality is a powerful marker of multiple diseases and the health status in a community. Life expectancy can be improved in communities through public health and clinical care practices. Reduced life expectancy has been associated with environmental risk factors, such as air, water, and soil pollution, noise, proximity to traffic and contaminated sites, and climate risks (droughts, floods, wildfires, and extreme heat days), among others.

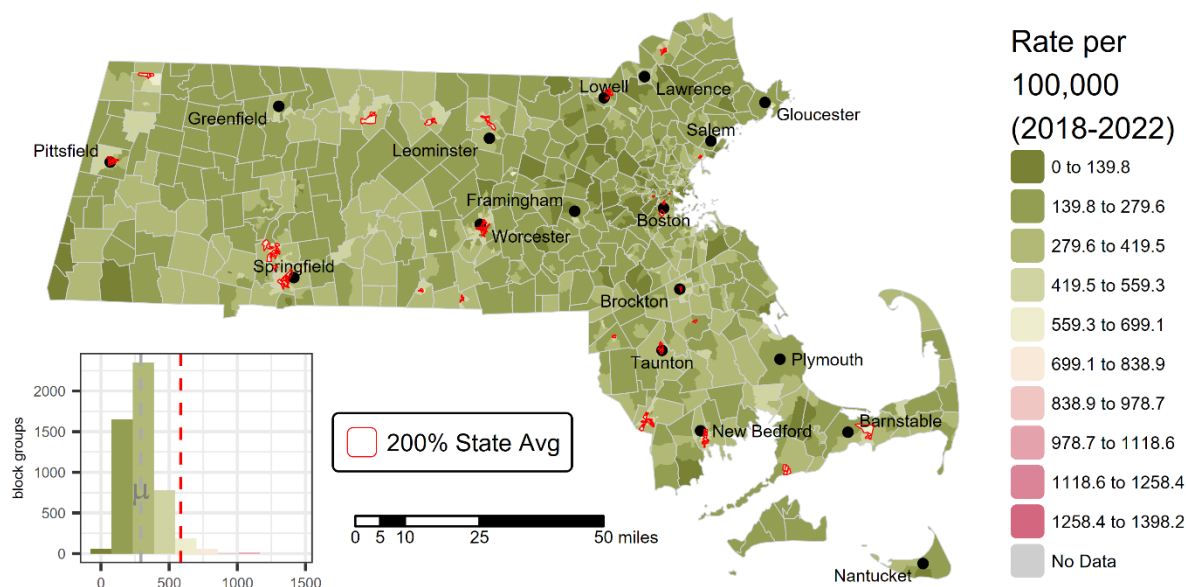
Method: Massachusetts Department of Public Health (MDPH) provided an age-adjusted premature mortality rate (PMR per 100,000) by census tract. All census block groups received the mortality rate for the census tract in which they are located.

Data years: 2018 – 2022

Originating geographic scale of data: census tracts

Source: Massachusetts Department of Public Health Death Data Registry of Vital Records and Statistics acquired via MassDEP Cumulative Impact Analysis in Air Quality Permitting at <https://www.mass.gov/info-details/cumulative-impact-analysis-in-air-quality-permitting#cia-guidance-and-tools->

Premature Mortality



Adult High Blood Pressure

Definition: Prevalence of high blood pressure among adults.

Description: High blood pressure, also called hypertension, is blood pressure that is higher than normal. Higher blood pressure levels put individuals at greater risk for other health problems, such as heart disease, heart attack, and stroke.

Indicator type: Sensitive Populations

Rationale: Hypertension, a type of cardiovascular disease (CVD) that leads to a prolonged increase in blood pressure, is a major risk factor for other CVD conditions like coronary heart disease and stroke. Risk factors for hypertension include obesity, physical inactivity, and high sodium consumption.^{xlii} Traditional risk factors for CVD, like male sex, older age, increased blood pressure, high cholesterol, and smoking, account for about 50 percent of cardiac events. There are also known environmental exposures that act independently or in conjunction with established risk factors to impact CVD and hypertension.^{xliii} Lead exposure can lead to both cardiovascular effects and cardiovascular-related mortality. There is consistent evidence to show that lead exposure increases blood pressure in adults, hypertension, and cardiovascular mortality, with more limited information on lead

relating to changes in heart rate variability and the development of atherosclerosis.^{xliv} Air pollution exposure has also been found to contribute to the development of CVD and hypertension and exacerbate existing CVD conditions. Evidence is particularly strong for short-term and long-term exposures to fine particulate matter, or particulate matter with a diameter of less than 2.5 μm .^{xlv} In addition, the effects of climate change may cause or worsen certain illnesses and health conditions. Climate-related changes to weather and heat can worsen air quality by impacting ozone and particulate matter concentrations, wildfires, and allergens. The effects of climate change on air quality pose human health risks, including heart disease and stroke. People with conditions such as hypertension are particularly vulnerable to the adverse effects of climate change on outdoor air quality. Certain populations remain at a disproportionate risk for exposure and negative health outcomes. Those at increased risk of exposure to particle pollution include non-white and low-income populations, as well as those who live or work in urban and industrial areas. Elderly and other high-risk populations are also heavily impacted. Studies have shown exposure to ambient airborne particulate matter could be associated with increased hospitalizations and mortality among older individuals, largely due to cardiopulmonary and cardiovascular disease.^{xlvi}

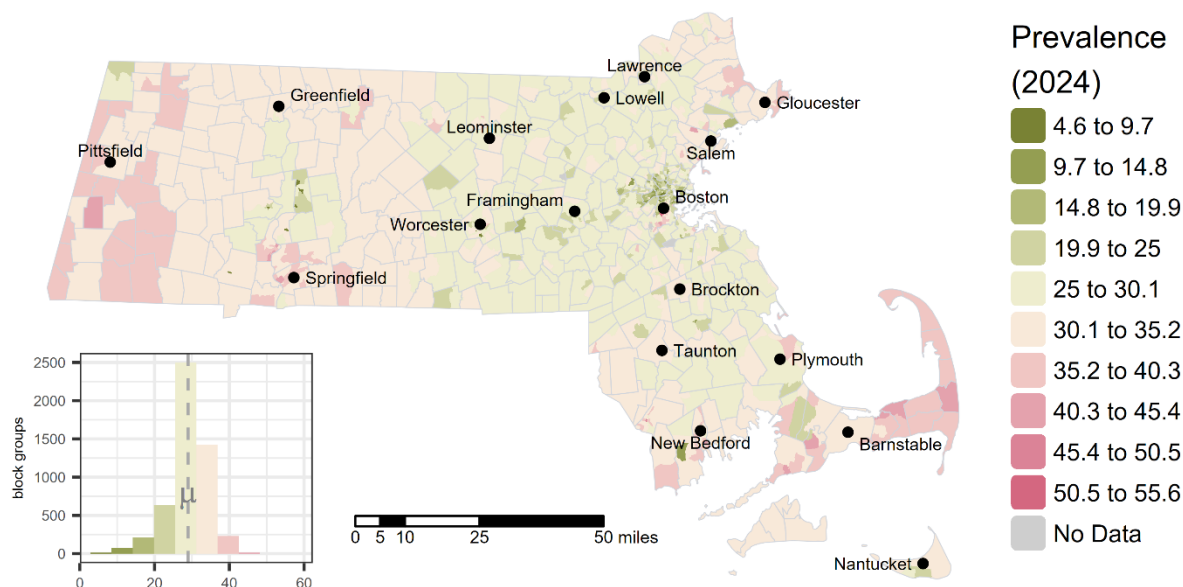
Method: Data provided a census tract level. All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2024

Originating geographic scale of data: census tracts

Source: CDC PLACES Health Outcomes at https://data.cdc.gov/500-Cities-Places/PLACES-Local-Data-for-Better-Health-Census-Tract-D/cwsq-ngmh/about_data

Adult High Blood Pressure



Coronary Heart Disease

Definition: Prevalence of coronary heart disease among adults.

Description: Coronary heart disease (also called ischemic heart disease) is caused by plaque buildup on the arteries. These blockages can limit the amount blood and oxygen the heart receives causing chest pain or angina. Coronary heart disease may present as an acute myocardial infarction which happens when the blood flow to a part of the heart is blocked by plaque.

Indicator type: Sensitive Populations

Rationale: Coronary artery disease (CAD) is the most common type of heart disease in the United States. Overweight, physical inactivity, unhealthy eating, and smoking tobacco are behavioral risk factors for CAD. There is a growing body of evidence showing that physicochemical factors in the environment contribute significantly to the cardiovascular diseases such as coronary heart disease. Urbanization is often associated with accumulation and intensification of these stressors.^{xlvi} Globally, some research concludes that environmental causes may be a larger cardiovascular mortality risk factor than those attributable to metabolic, tobacco use, and behavioral risk factors.^{xlvi}

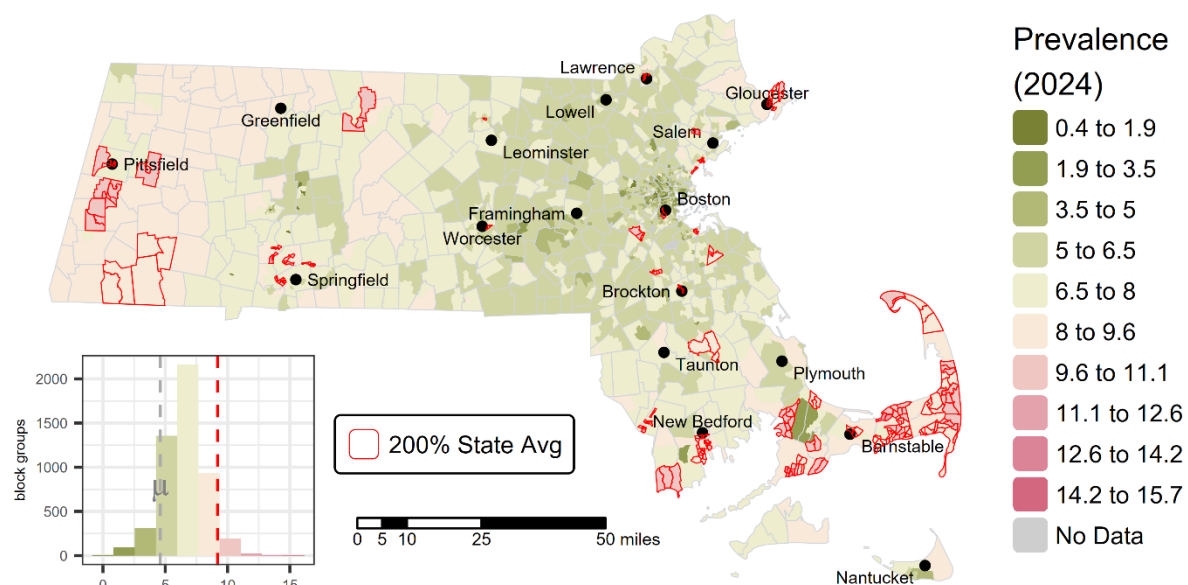
Method: Data provided a census tract level. All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2024

Originating geographic scale of data: census tracts

Source: CDC PLACES Health Outcomes at https://data.cdc.gov/500-Cities-Places/PLACES-Local-Data-for-Better-Health-Census-Tract-D/cwsq-ngmh/about_data

Coronary Heart Disease



Chronic Obstructive Pulmonary Disease (COPD)

Definition: Prevalence of chronic obstructive pulmonary disease among adults.

Description: Chronic Obstructive Pulmonary Disease, or COPD, refers to a group of diseases including emphysema and chronic bronchitis, that block airflow and cause breathing-related problems.

Indicator type: Sensitive Populations

Rationale: COPD is a leading cause of death in the US and in Massachusetts. The main cause of COPD is smoking, but nonsmokers can also get COPD. Approximately 85-90% of COPD diagnoses are attributed to cigarette smoking. Occupation exposures may account

for another 15% of COPD diagnoses. A history of asthma may increase the risk of developing COPD. Long-term exposure to air pollution, secondhand smoke, dust, fumes and chemicals (which are often work-related) can cause COPD.^{xlix} Although the primary cause of COPD is smoking, an increasing number of studies have reported associations between indoor and outdoor air pollution exposures and COPD, suggesting that environmental exposure could be driving a large percentage of COPD cases.

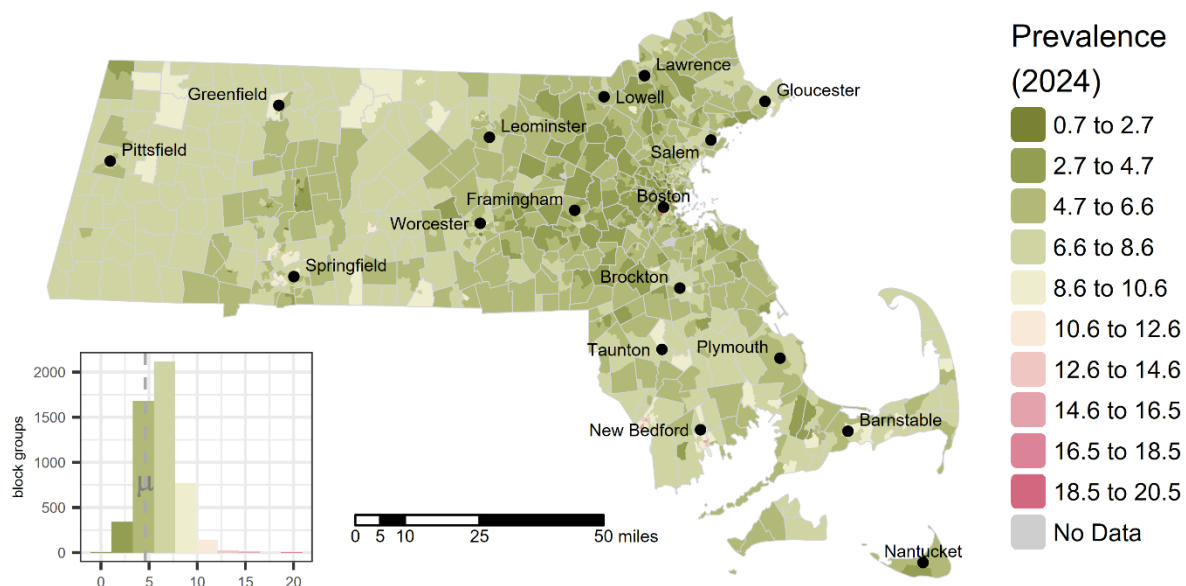
Method: Data provided a census tract level. All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2024

Originating geographic scale of data: census tracts

Source: CDC PLACES Health Outcomes at https://data.cdc.gov/500-Cities-Places/PLACES-Local-Data-for-Better-Health-Census-Tract-D/cwsq-ngmh/about_data

Chronic Obstructive Pulmonary Disease (COPD)



Adult Cancer

Definition: Prevalence of cancer (non-skin) or melanoma among adults.

Description: Cancer is a group of similar diseases in which some of the body's cells grow uncontrollably and spread to other parts of the body. There are many types of cancer that affect different parts of the body. At least two-thirds of cancer cases are caused by external factors. These factors include tobacco and alcohol use, diet, sun exposure, and certain infections. Exposure to certain chemical pollutants or radiation is also a risk factor. This indicator represents how many people are living with cancer.

Indicator type: Sensitive Populations

Rationale: Cancer is the leading cause of death in Massachusetts.ⁱ Cancer is a group of chronic diseases that can start in any organ or tissue of the human body. Cancers are also called malignant neoplasms or tumors. Cancer develops when abnormal cells grow uncontrollably and go beyond their usual boundaries to invade adjoining body parts or spread to other organs. Lung, colorectal, and breast cancers are common types of cancers. Cancer can happen in all age groups. Among 30-50% of all cancers can be prevented.ⁱⁱ Multiple environmental risk factors have been associated with cancer development, such as air, soil, and water pollution, metals, pesticides, sun exposure, chemicals, and radon, among others.ⁱⁱⁱ In those patients diagnosed with cancer, environmental pollutants can worsen the prognosis.

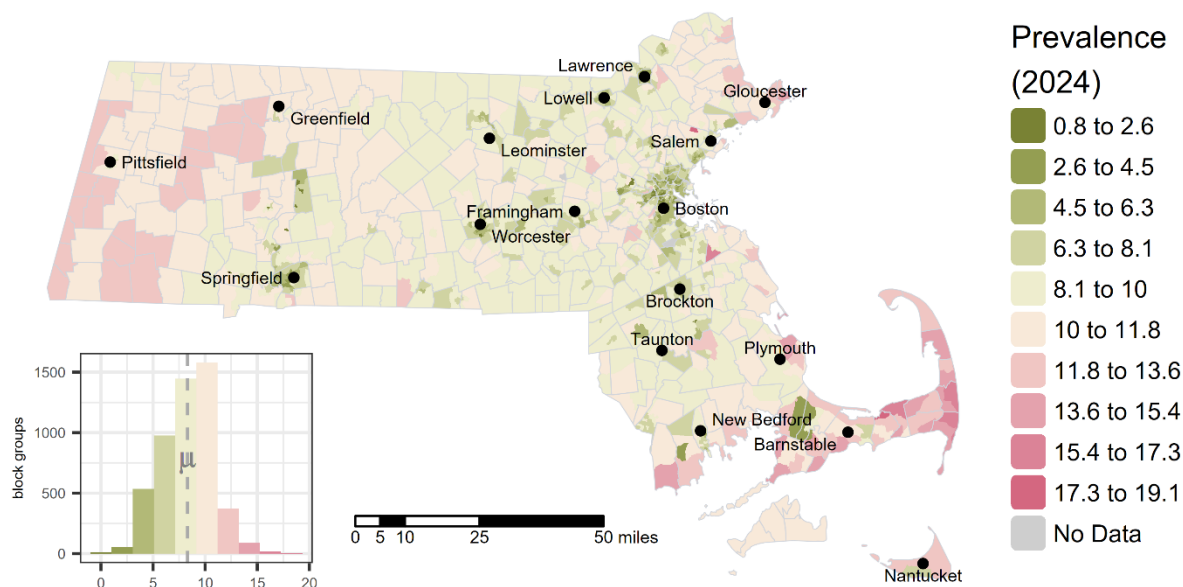
Method: Data provided at census tract level. All census block groups received the prevalence value for the census tract in which they are located.

Data years: 2024

Originating geographic scale of data: census tracts

Source: CDC PLACES Health Outcomes at https://data.cdc.gov/500-Cities-Places/PLACES-Local-Data-for-Better-Health-Census-Tract-D/cwsq-ngmh/about_data

Adult Cancer



Population Characteristics - Socioeconomic Factors

Adults Without a High School Diploma

Definition: Percent of people age 25 or older whose education is less than a high school diploma.

Description: How much education a person has is associated with wealth, health, length of life, and exposure to pollution. People with less education are more likely to be affected by pollution because of where they live or work.

Indicator type: Socioeconomic Factors

Rationale: Educational attainment is an important independent predictor of health.^{lviii}

Individuals with lower education in the US have a lower life expectancy,^{liv} are more likely to be obese,^{lv} and are more likely to experience psychiatric disorders compared to individuals with higher education.^{lvi} Education is often inversely related to the degree of exposure to indoor and outdoor pollution. Several studies have associated educational attainment with susceptibility to the health impacts of environmental pollutants. For example, individuals without a high school education appear to be at higher risk of mortality associated with particulate air pollution than those with a high school education.^{lvii} There is also evidence

that the effects of air and traffic-related pollution on respiratory illness, including childhood asthma, are more severe in communities with lower levels of education.^{lviii} In studies evaluating air pollution related risks of adverse birth outcomes, mothers with low educational attainment were found to be more vulnerable.^{lix} While there is a positive association between educational attainment and health, racial and ethnic minorities gain fewer health benefits from educational attainment than Whites.^{lx}

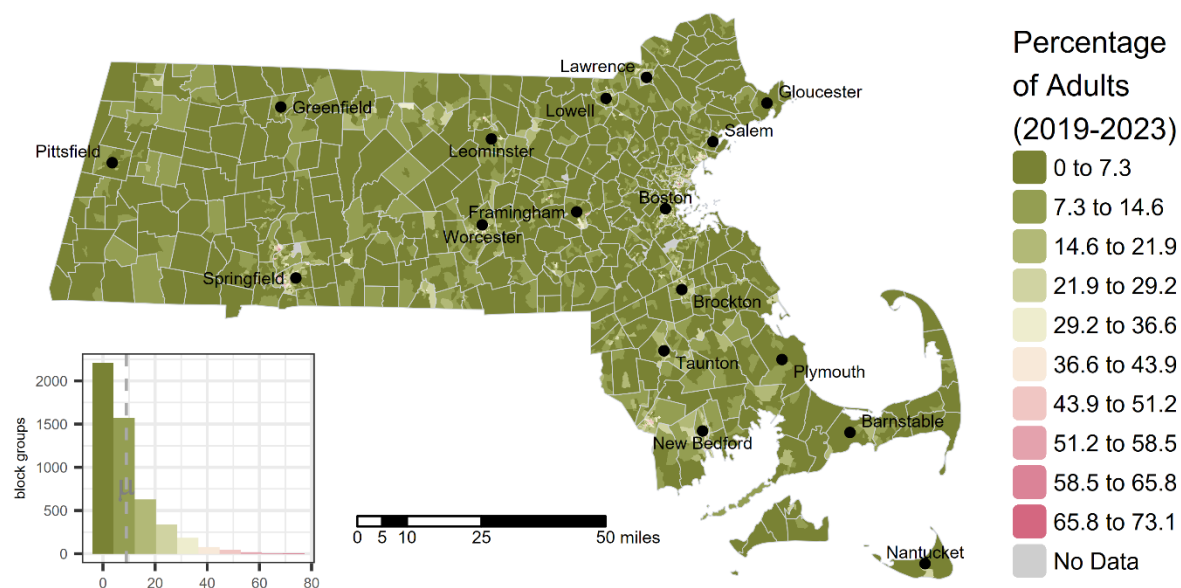
Method: The ACS education information is captured in the table Educational Attainment for the Population 25 Years and Over (ACS Table ID: B15003) at the census block group level. Percentage with less than a high school education is calculated as the sum of adults with 12th grade education but no diploma, divided by the total number of adults age 25 and older.

Data years: 2019 – 2023

Originating geographic scale of data: census block groups

Source: US American Community Survey 5-year Estimates for 2019 – 2023.

Adults without a High School Diploma



Housing Burdened Low Income Households:

Definition: Percent of households that are both low income (making less than 80% of the HUD Area Median Family Income) and severely burdened by housing costs (paying greater than 50% of their income to housing costs).

Description: Areas where low-income households may be stressed by high housing costs can be identified through the Housing and Urban Development (HUD) Comprehensive Housing Affordability Strategy (CHAS) data. This indicator measures households earning less than 80% of HUD Area Median Family Income by county and paying greater than 50% of their income to housing costs. The indicator takes into account the regional cost of living for both homeowners and renters, and incorporates the cost of utilities. CHAS data are calculated from US Census Bureau's American Community Survey (ACS).

Indicator type: Socioeconomic Factors

Rationale: Housing cost-burdened communities are impacted by a reduced economic capacity to satisfy basic needs. Economic capacity also impacts environmental exposures, household quality, and environmental health resilience (e.g., health status, lifestyle, access to health services).^{lx} Housing cost-burdened communities tend to live in proximity to areas with higher environmental hazards, such as industrial sites, waste management facilities, and high-traffic density areas. Environmental inequities in housing cost-burdened communities are important, especially when other social factors converge (e.g., communities of color, lower levels of education).

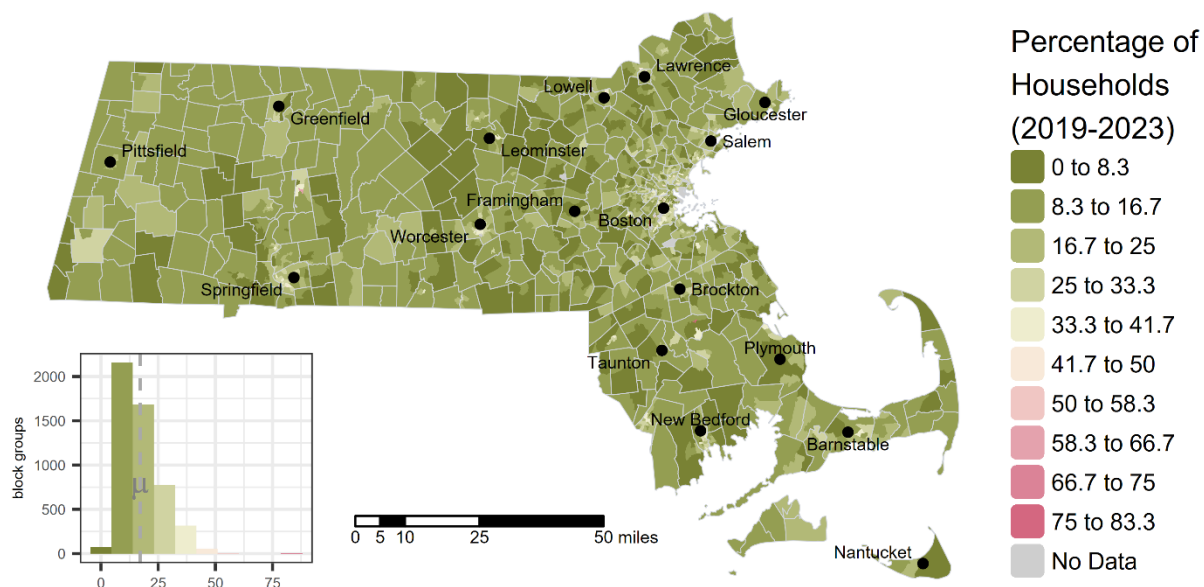
Method: Data provided at census tract level. All census block groups received the percentage value for the census tract in which they are located.

Data years: 2019 – 2023

Originating geographic scale of data: census tract

Source: US Department of Housing and Urban Development CHAS (Comprehensive Housing Affordability Strategy) at <https://www.huduser.gov/portal/datasets/cp.html>

Housing Burdened Low Income Households



Linguistic Isolation

Definition: Percentage of limited English-speaking households.

Description: A limited English-speaking household is defined as a household in which no one age 14 or over speaks English at least "very well" as reported in the U.S. Census Bureau's ACS. Limited English proficiency affects how well people can access services and information in their community, such as public notices about environmental contamination. Linguistic isolation describes individuals and households that have limited English proficiency or speak languages other than English at home.

Indicator type: Socioeconomic Factors

Rationale: Linguistic isolation indicates communities that could be less resilient to climate and environmental risks.^{lxiii} Those communities might be less informed about environmental risks (e.g., air quality alerts or water violations) and climate risks (e.g., heat, flood, drought, and wildfire alerts) when information about those risks is only made available in English. Furthermore, linguistically isolated households may also have less access to health information (e.g., healthy lifestyles or disease screening), health care services (e.g., insurance or disease treatment), and environmental health actions (e.g., incentives for housing weatherization or pollution clean-ups). Linguistic isolation is highly

correlated with income, and as mentioned in the low-income indicator, income impacts environmental exposures, household quality, and environmental health resilience.

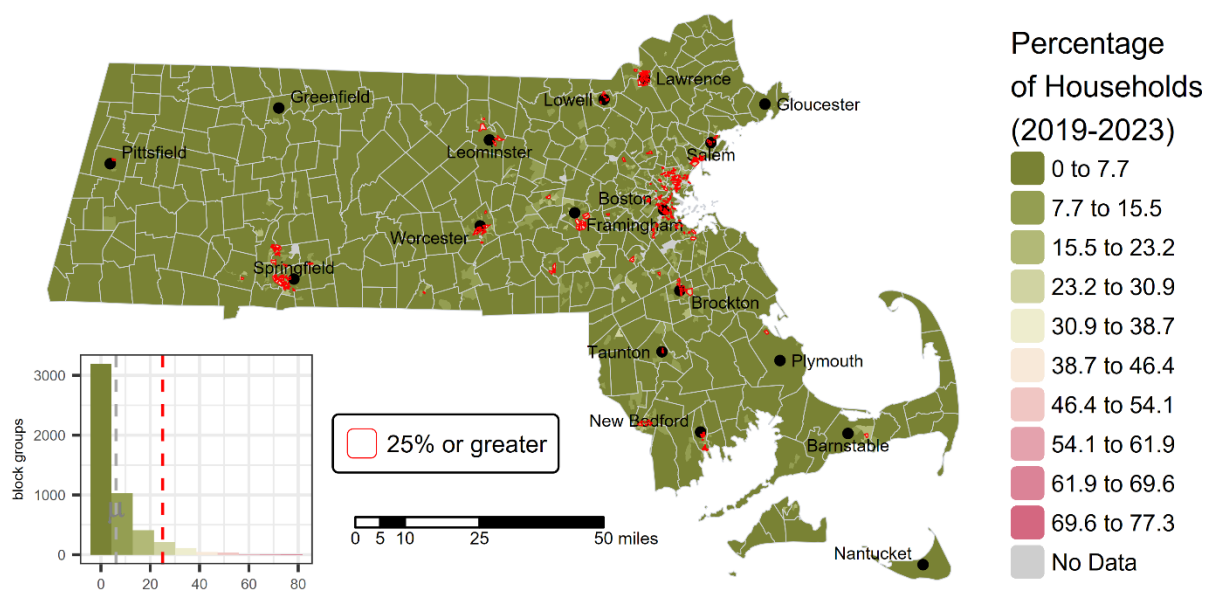
Method: The ACS limited English speaking household information is captured in the table Household Language by Household Limited English Speaking Status (ACS Table ID: C16002) at the census block group level. Percentage limited English speaking households is computed as the sum of limited English speaking households whose primary language spoken at home is Spanish, Other Indo-European languages, Asian/Pacific Island languages, and other languages, divided by the total number of households.

Data years: 2019 – 2023

Originating geographic scale of data: census block groups

Source: US American Community Survey 5-year Estimates for 2019 – 2023

Linguistic Isolation



Poverty

Definition: Percent of households whose income is less than or equal to twice the poverty level.

Description: Income is strongly associated with health outcomes, environmental exposures, and access to, or quality of services. This indicator describes how many people in a community live at or below twice the federal poverty level. The poverty level is a national number and the same across all geographic regions. To accommodate differences in the varying costs of living across the United States and other factors, analysts typically use twice the poverty level to capture low-income households, especially in high-cost areas such as Massachusetts. In 2023, twice the federal poverty threshold meant an annual household income of \$62,400 or less for a family of four.

Indicator type: Socioeconomic Factors

Rationale: Income correlates with access to better services and goods and is associated with better health outcomes and life expectancy. Income is also associated with housing quality and climate resilience. Low-income communities tend to live in areas with more environmental pollutants (e.g., industrial sites, waste management facilities, high-traffic areas) and climate hazards (heat islands, floods, droughts, and wildfires). Moreover, low income communities also suffer from higher disease rates, injuries, and premature mortality and have less access to public health and health care opportunities.^{lxiii}

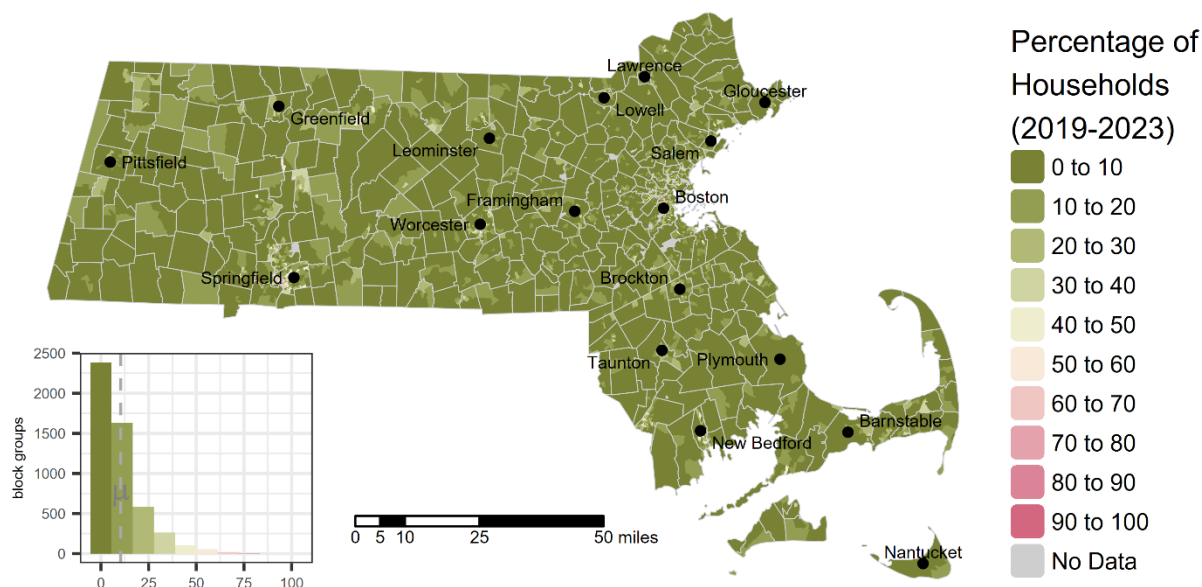
Method: The ACS low income information is captured in the table Ratio of Income to Poverty Level in the Past 12 Months (ACS Table ID: C17002) by census block group.

Data years: 2019 – 2023

Originating geographic scale of data: census block groups

Source: US American Community Survey 5-year Estimates for 2019 – 2023.

Poverty



Unemployment

Definition: Percentage of the population over the age of 16 that who are unemployed and eligible for the labor force. Excludes retirees, students, homemakers, institutionalized persons except prisoners, those not looking for work, and military personnel on active duty.

Description: Unemployment impacts people’s health, well-being, and quality-of-life. Many people live in poverty because they cannot find employment. In addition, some workforce participants are “underemployed”, including involuntary part-time employment, poverty-wage employment, and/or insecure employment. In either case, these people struggle to afford healthy foods, health care, and safe, affordable housing, and lack the time and resources for a healthy lifestyle (e.g., exercise, meditation, stress reduction).

Indicator type: Socioeconomic Factors

Rationale: Unemployment has a wide range of effects on health which contribute to the burden placed on vulnerable communities. It has been shown to negatively impact mental and physical health. Higher rates of unemployment are associated with overall mortality, as well as mortality specifically due to transport accidents, poisonings (which include drug overdoses), and suicides.^{lxiv} Unemployment is also associated with increases in physical morbidity as well as mortality.

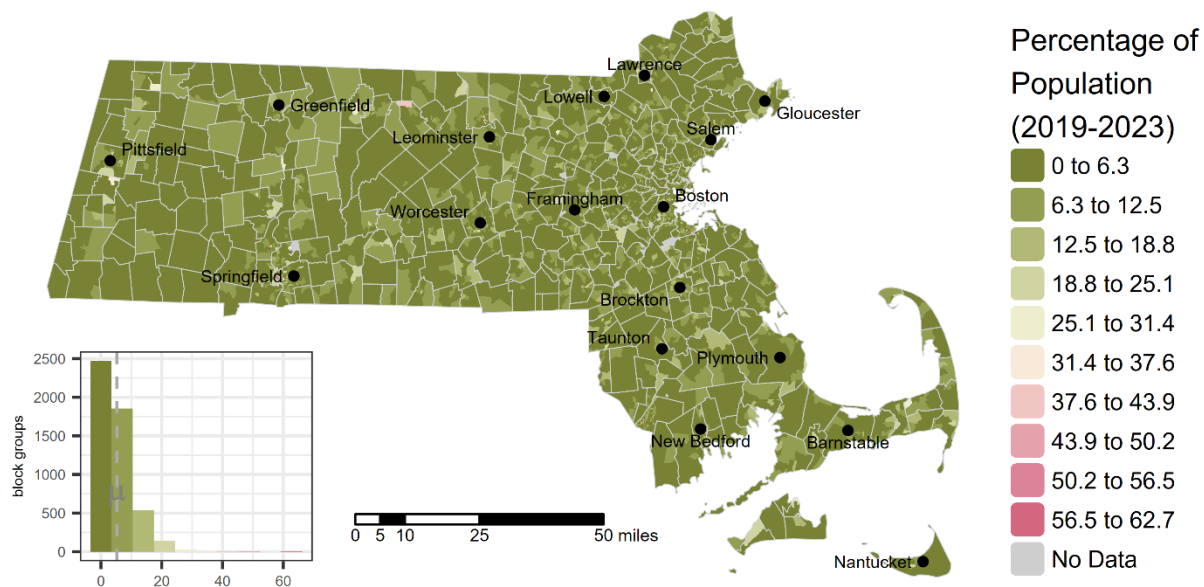
Method: The ACS unemployment information is captured in the table Employment Status for the Population 16 Years and Over (Table ID: B23025).

Data years: 2019 – 2023

Originating geographic scale of data: census block groups

Source: US American Community Survey 5-year Estimates for 2019 – 2023.

Unemployment



ⁱ US EPA, “Particulate Matter (PM) Basics,” Overviews and Factsheets, April 19, 2016, <https://www.epa.gov/pm-pollution/particulate-matter-pm-basics>.

ⁱⁱ American Lung Association, “Particle Pollution,” accessed July 18, 2025, <https://www.lung.org/clean-air/outdoors/what-makes-air-unhealthy/particle-pollution>; Cheng-Kuan Lin et al., “Association between Exposure to Ambient Particulate Matters and Risks of Autism Spectrum Disorder in Children: A Systematic Review and Exposure-Response Meta-Analysis,” *Environmental Research Letters* 16, no. 6 (2021): 063003, <https://doi.org/10.1088/1748-9326/abfcf7>; David Rojas-Rueda et al., “Environmental Risk Factors and Health: An Umbrella Review of Meta-Analyses,” *International Journal of Environmental Research and Public Health* 18, no. 2 (2021): 2, <https://doi.org/10.3390/ijerph18020704>.

ⁱⁱⁱ American Lung Association, “Ozone,” accessed July 18, 2025, <https://www.lung.org/clean-air/outdoors/what-makes-air-unhealthy/ozone>.

^{iv} US EPA, “Ground-Level Ozone Pollution,” Other Policies and Guidance, May 15, 2015, <https://www.epa.gov/ground-level-ozone-pollution>.

^v American Lung Association, “Nitrogen Dioxide,” accessed July 18, 2025, <https://www.lung.org/clean-air/outdoors/what-makes-air-unhealthy/nitrogen-dioxide>.

^{vi} US EPA, “Basic Information about NO₂,” Overviews and Factsheets, July 6, 2016, <https://www.epa.gov/no2-pollution/basic-information-about-no2>.

-
- vii Laura August et al., *CalEnviroScreen 4.0 Report* (California Environmental Protection Agency, 2021); EPA, “Technical Documentation for EJScreens,” U.S. Environmental Protection Agency, 2019, <https://www.epa.gov/ejscreen/technical-documentation-ejscreen>.
- viii American Lung Association, “Particle Pollution.”
- ix US EPA, *EJScreens Technical Documentation for Version 2.3* (US Environmental Protection Agency, 2024), <https://www.epa.gov/ejscreen/download-ejscreen-data>.
- x Shannon H. Gaffney et al., “Influence of Geographic Location in Modeling Blood Pesticide Levels in a Community Surrounding a U.S. Environmental Protection Agency Superfund Site,” *Environmental Health Perspectives* 113, no. 12 (2005): 1712–16, <https://doi.org/10.1289/ehp.8154>.
- xi Ami R. Zota et al., “Metal Sources and Exposures in the Homes of Young Children Living near a Mining-Impacted Superfund Site,” *Journal of Exposure Science & Environmental Epidemiology* 21, no. 5 (2011): 495–505, <https://doi.org/10.1038/jes.2011.21>.
- xii Shiwen Li et al., “Examining Disparities in PFAS Plasma Concentrations: Impact of Drinking Water Contamination, Food Access, Proximity to Industrial Facilities and Superfund Sites,” *Environmental Research* 264 (January 2025): 120370, <https://doi.org/10.1016/j.envres.2024.120370>.
- xiii Paul J. Squillace and Michael J. Moran, “Factors Associated with Sources, Transport, and Fate of Volatile Organic Compounds and Their Mixtures in Aquifers of the United States,” *Environmental Science & Technology* 41, no. 7 (2007): 2123–30, <https://doi.org/10.1021/es061079w>.
- xiv Michael J. Moran et al., “Chlorinated Solvents in Groundwater of the United States,” *Environmental Science & Technology* 41, no. 1 (2007): 74–81, <https://doi.org/10.1021/es061553y>.
- xv Sara Picone et al., “Sensitivity Analysis on Parameters and Processes Affecting Vapor Intrusion Risk,” *Environmental Toxicology and Chemistry* 31, no. 5 (2012): 1042–52, <https://doi.org/10.1002/etc.1798>; Yijun Yao et al., “Examination of the Influence of Environmental Factors on Contaminant Vapor Concentration Attenuation Factors Using the U.S. EPA’s Vapor Intrusion Database,” *Environmental Science & Technology* 47, no. 2 (2013): 906–13, <https://doi.org/10.1021/es303441x>.
- xvi C Pellerin and S M Booker, “Reflections on Hexavalent Chromium: Health Hazards of an Industrial Heavyweight,” *Environmental Health Perspectives* 108, no. 9 (2000): A402–7, <https://doi.org/10.1289/ehp.108-a402>.
- xvii M Vrijheid, “Health Effects of Residence near Hazardous Waste Landfill Sites: A Review of Epidemiologic Literature,” *Environmental Health Perspectives* 108, no. suppl 1 (2000): 101–12, <https://doi.org/10.1289/ehp.00108s1101>.
- xviii Maria Kouznetsova et al., “Increased Rate of Hospitalization for Diabetes and Residential Proximity of Hazardous Waste Sites,” *Environmental Health Perspectives* 115, no. 1 (2007): 75–79, <https://doi.org/10.1289/ehp.9223>; Alexander V. Sergeev and David O. Carpenter, “Hospitalization Rates for Coronary Heart Disease in Relation to Residence Near Areas Contaminated with Persistent Organic Pollutants and Other Pollutants,” *Environmental Health Perspectives* 113, no. 6 (2005): 756–61, <https://doi.org/10.1289/ehp.7595>.
- xix X. F. Lou and J. Nair, “The Impact of Landfilling and Composting on Greenhouse Gas Emissions – A Review,” *Bioresource Technology*, Selected papers from the International Conference on Technologies and Strategic Management of Sustainable Biosystems, vol. 100, no. 16 (2009): 3792–98, <https://doi.org/10.1016/j.biortech.2008.12.006>; Joseph Ofungwu and Steven Eget, “Brownfields and Health Risks—Air Dispersion Modeling and Health Risk Assessment at Landfill Redevelopment Sites,” *Integrated Environmental Assessment and Management* 2, no. 3 (2006): 253–61, <https://doi.org/10.1002/ieam.5630020305>.
- xx FEMA, *Landfill Fires: Their Magnitude, Characteristics, and Mitigation*, FA-225 (US Fire Administration, 2002), <https://www.sustainable-design.ie/fire/FEMA-LandfillFires.pdf>.
- xxi Christopher D. Heaney et al., “Relation between Malodor, Ambient Hydrogen Sulfide, and Health in a Community Bordering a Landfill,” *Environmental Research* 111, no. 6 (2011): 847–52, <https://doi.org/10.1016/j.envres.2011.05.021>.
- xxii CDC, “Drought and Your Health,” Drought and Health, February 14, 2025, <https://www.cdc.gov/drought-health/about/index.html>.
- xxiii FEMA, “Wildfire,” accessed July 22, 2025, <https://resilience-fema.hub.arcgis.com/pages/wildfire>.

-
- ^{xxiv} CDC, “How Wildfire Smoke Affects Your Body,” Wildfires, February 20, 2025, <https://www.cdc.gov/wildfires/risk-factors/index.html>.
- ^{xxv} “Flood Zones | FEMA.Gov,” July 8, 2020, <https://www.fema.gov/about/glossary/flood-zones>.
- ^{xxvi} FEMA, “What Is My Flood Risk | The National Flood Insurance Program,” accessed July 22, 2025, <https://www.floodsmart.gov/flood-zones-and-maps/what-is-my-flood-risk>.
- ^{xxvii} Chris Dolce, “America’s Second-Biggest Weather Killer Is Flooding, And Texas Leads The Way | Weather.Com,” July 7, 2025, <https://weather.com/safety/floods/news/2025-07-07-flooding-united-states-second-biggest-weather-killer-texas>; FEMA, “What Is My Flood Risk | The National Flood Insurance Program.”
- ^{xxviii} CDC, “Floods and Your Safety,” Floods, September 30, 2024, <https://www.cdc.gov/floods/about/index.html>.
- ^{xxix} Kristie L. Ebi et al., “Hot Weather and Heat Extremes: Health Risks,” *The Lancet* 398, no. 10301 (2021): 698–708, [https://doi.org/10.1016/S0140-6736\(21\)01208-3](https://doi.org/10.1016/S0140-6736(21)01208-3); World Health Organization, “Heat and Health,” accessed July 22, 2025, <https://www.who.int/news-room/fact-sheets/detail/climate-change-heat-and-health>.
- ^{xxx} Ebi et al., “Hot Weather and Heat Extremes”; World Health Organization, “Heat and Health.”
- ^{xxxi} MA Department of Public Health, “Asthma Overview,” accessed July 22, 2025, <https://matracking.ehs.state.ma.us/Health-Data/Asthma/index.html>.
- ^{xxxii} US EPA, *Indicators of Environmental Health Disparities: Childhood Asthma Prevalence*, EPA 231R24003 (US Environmental Protection Agency, 2024), <https://www.epa.gov/system/files/documents/2024-12/ej-indicators-asthma.pdf>.
- ^{xxxiii} World Health Organization, “Lead Poisoning,” accessed July 22, 2025, <https://www.who.int/news-room/fact-sheets/detail/lead-poisoning-and-health>.
- ^{xxxiv} CDC, “About Childhood Lead Poisoning Prevention,” Childhood Lead Poisoning Prevention, March 13, 2025, <https://www.cdc.gov/lead-prevention/about/index.html>.
- ^{xxxv} CDC, “Preterm Birth,” Maternal Infant Health, February 24, 2025, <https://www.cdc.gov/maternal-infant-health/preterm-birth/index.html>.
- ^{xxxvi} CDC, “Reproductive & Birth Outcomes,” Environmental Public Health Tracking, May 29, 2024, <https://www.cdc.gov/environmental-health-tracking/php/data-research/reproductive-birth-outcomes.html>.
- ^{xxxvii} US EPA, *Adverse Birth Outcomes* (US Environmental Protection Agency, 2015), <https://www.epa.gov/sites/default/files/2015-06/documents/health-adverse-birth-outcomes.pdf>.
- ^{xxxviii} NCHS, *National Vital Statistics System Improvements Fact Sheet* (National Center for Health Statistics, 2019), <https://www.cdc.gov/nchs/data/factsheets/factsheet-nvss-improvements-H.pdf>.
- ^{xxxix} US EPA, “Integrated Science Assessment (ISA) for Particulate Matter,” Reports and Assessments, US EPA, November 20, 2019, <https://assessments.epa.gov/risk/document/&deid%3D347534>; US EPA, “Integrated Science Assessment (ISA) for Oxides of Nitrogen,” Reports and Assessments, US EPA, 2016, https://ordspub.epa.gov/ords/eims/eimscomm.getfile?p_download_id=526855; US EPA, “Integrated Science Assessment (ISA) for Carbon Monoxide (Final Report, Jan 2010),” Reports and Assessments, US EPA, January 13, 2010, <https://assessments.epa.gov/isa/document/&deid%3D218686>.
- ^{xl} US EPA, “Integrated Science Assessment (ISA) for Lead (Final Report),” Reports and Assessments, US EPA, November 20, 2023, <https://assessments.epa.gov/isa/document/&deid%3D359536>.
- ^{xli} US EPA, *Adverse Birth Outcomes*.
- ^{xlii} CDC, “About High Blood Pressure,” High Blood Pressure, January 28, 2025, <https://www.cdc.gov/high-blood-pressure/about/index.html>.
- ^{xliii} US EPA, *Indicators of Environmental Health Disparities Age-Adjusted Hypertension Technical Documentation* (US Environmental Protection Agency, 2024).
- ^{xliv} US EPA, “Integrated Science Assessment (ISA) for Lead (Final Report).”
- ^{xlv} US EPA, “Particle Pollution and Cardiovascular Effects,” Collections and Lists, September 15, 2014, <https://www.epa.gov/pmcourse/particle-pollution-and-cardiovascular-effects>.
- ^{xlvi} US EPA, *Indicators of Environmental Health Disparities Age-Adjusted Hypertension Technical Documentation*.
- ^{xlvii} Thomas Münzel et al., “Environmental Risk Factors and Cardiovascular Diseases: A Comprehensive Expert Review,” *Cardiovascular Research* 118, no. 14 (2022): 2880–902, <https://doi.org/10.1093/cvr/cvab316>.

-
- xlvi Shilpa S. Shetty et al., “Environmental Pollutants and Their Effects on Human Health,” *Heliyon* 9, no. 9 (2023), <https://doi.org/10.1016/j.heliyon.2023.e19496>.
- xlix Massachusetts Environmental Public Health Tracking, “Chronic Obstructive Pulmonary Disease (COPD),” accessed July 23, 2025, <https://matracking.ehs.state.ma.us/Health-Data/copd.html>.
- ⁱ Massachusetts Executive Office of Health and Human Services, “Mortality | Mass.Gov,” accessed July 23, 2025, <https://www.mass.gov/info-details/mortality>.
- ⁱⁱ World Health Organization, “Preventing Cancer,” accessed July 23, 2025, <https://www.who.int/activities/preventing-cancer>.
- ⁱⁱⁱ National Cancer Institute, “Cancer-Causing Substances in the Environment - NCI,” *cgvArticle*, March 18, 2015, [nciglobal.ncienterprise, https://www.cancer.gov/about-cancer/causes-prevention/risk/substances](https://www.cancer.gov/about-cancer/causes-prevention/risk/substances).
- ⁱⁱⁱⁱ David M. Cutler and Adriana Lleras-Muney, *Education and Health: Evaluating Theories and Evidence*, no. w12352 (National Bureau of Economic Research, 2006), <https://doi.org/10.3386/w12352>; Anna Zajacova and Elizabeth M. Lawrence, “The Relationship Between Education and Health: Reducing Disparities Through a Contextual Approach,” *Annual Review of Public Health* 39, no. Volume 39, 2018 (2018): 273–89, <https://doi.org/10.1146/annurev-publhealth-031816-044628>.
- ^{liv} Jennifer Karas Montez and Erin M. Bisesti, “Widening Educational Disparities in Health and Longevity,” *Annual Review of Sociology* 50, no. Volume 50, 2024 (2024): 547–64, <https://doi.org/10.1146/annurev-soc-071723-080605>.
- ^{lv} A. K. Cohen et al., “Educational Attainment and Obesity: A Systematic Review,” *Obesity Reviews* 14, no. 12 (2013): 989–1005, <https://doi.org/10.1111/obr.12062>.
- ^{lvi} Julie Erickson et al., “Educational Attainment as a Protective Factor for Psychiatric Disorders: Findings from a Nationally Representative Longitudinal Study,” *Depression and Anxiety* 33, no. 11 (2016): 1013–22, <https://doi.org/10.1002/da.22515>.
- ^{lvii} C. Arden Pope et al., “Mortality Risk and Fine Particulate Air Pollution in a Large, Representative Cohort of U.S. Adults,” *Environmental Health Perspectives* 127, no. 7 (2019): 077007, <https://doi.org/10.1289/EHP4438>.
- ^{lviii} Sabit Cakmak et al., “Respiratory Health Effects of Air Pollution Gases: Modification by Education and Income,” *Archives of Environmental & Occupational Health* 61, no. 1 (2006): 5–10, <https://doi.org/10.3200/AEOH.61.1.5-10>; Matthew J. Neidell, “Air Pollution, Health, and Socio-Economic Status: The Effect of Outdoor Air Quality on Childhood Asthma,” *Journal of Health Economics* 23, no. 6 (2004): 1209–36, <https://doi.org/10.1016/j.jhealeco.2004.05.002>; Ketan Shankardass et al., “Parental Stress Increases the Effect of Traffic-Related Air Pollution on Childhood Asthma Incidence,” *Proceedings of the National Academy of Sciences* 106, no. 30 (2009): 12406–11, <https://doi.org/10.1073/pnas.0812910106>.
- ^{lix} Neil Thayamballi et al., “Impact of Maternal Demographic and Socioeconomic Factors on the Association Between Particulate Matter and Adverse Birth Outcomes: A Systematic Review and Meta-Analysis,” *Journal of Racial and Ethnic Health Disparities* 8, no. 3 (2021): 743–55, <https://doi.org/10.1007/s40615-020-00835-2>.
- ^{lx} Shervin Assari, “Blacks’ Diminished Return of Education Attainment on Subjective Health; Mediating Effect of Income,” *Brain Sciences* 8, no. 9 (2018): 9, <https://doi.org/10.3390/brainsci8090176>; Caryn N. Bell et al., “Racial Non-Equivalence of Socioeconomic Status and Self-Rated Health among African Americans and Whites,” *SSM - Population Health* 10 (April 2020): 100561, <https://doi.org/10.1016/j.ssmph.2020.100561>.
- ^{lxi} US Department of Health and Human Services, “Social Determinants of Health - Healthy People 2030 | Odphp.Health.Gov,” accessed July 23, 2025, <https://odphp.health.gov/healthypeople/priority-areas/social-determinants-health>.
- ^{lxii} US Department of Health and Human Services, “Social Determinants of Health - Healthy People 2030 | Odphp.Health.Gov.”
- ^{lxiii} US Department of Health and Human Services, “Social Determinants of Health - Healthy People 2030 | Odphp.Health.Gov.”
- ^{lxiv} Sarah H. Gordon and Benjamin D. Sommers, “Recessions, Poverty, and Mortality in the United States: 1993–2012,” *American Journal of Health Economics* 2, no. 4 (2016): 489–510, https://doi.org/10.1162/AJHE_a_00060; Karsten I. Paul and Klaus Moser, “Unemployment Impairs Mental Health: Meta-Analyses,” *Journal of Vocational Behavior* 74, no. 3 (2009): 264–82, <https://doi.org/10.1016/j.jvb.2009.01.001>; Christopher J. Ruhm, “Recessions, Healthy No More?,” *Journal of Health Economics* 42 (July 2015): 17–28, <https://doi.org/10.1016/j.jhealeco.2015.03.004>.